

Hidden Tax & Wealth Strategies for Orthopedic Surgeons Approaching Retirement

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Executive Summary

Orthopedic surgeons in their peak earning years face a **steep tax burden** that can quietly erode their wealth as retirement nears. Earning between \$700,000 and \$2 million annually, many surgeons pay effective tax rates of 40% or more ¹ ², especially in high-tax states. In fact, the average full-time orthopedic surgeon earns around **\$745,000** (with top earners at \$2 million) ³, placing them in the highest tax brackets. Yet **75% of physicians feel they overpay in taxes** ⁴, and nearly half (48%) **prepare their own taxes** without specialized help ⁵ – a recipe for missed opportunities. This guide addresses that gap, revealing **hidden tax and wealth strategies** tailored for orthopedic surgeons aged 50–65 who are 5–10 years from retirement or practice exit.

The central message: **proactive tax planning in your final working years can dramatically boost your retirement lifestyle and legacy**. High-income surgeons often leave six-figure sums on the table by not leveraging advanced strategies in entity structuring, retirement plans, investment allocation, practice sale planning, and estate optimization. The cost of doing nothing is not just a higher tax bill – it's potentially working longer than desired or scaling back retirement goals. For example, a surgeon who reduces their tax outflow by **\$100,000 per year** in the last decade before retirement could accumulate roughly **\$1+ million** in extra savings, translating to an additional **\$40,000–\$50,000** in yearly retirement income (assuming moderate growth and a 4% withdrawal rate). Conversely, failing to plan might mean retiring with significantly less. About **20% of physicians retire with over \$5 million saved** ⁶ – roughly enough to generate a comfortable \$200–250K annual income. Without strategic tax savings, reaching that level may require additional years in the OR or settling for a leaner lifestyle.

This guide is organized into five core strategy sections, each addressing a critical area where mid-career and late-career surgeons can find hidden savings:

- **Entity & Income Structuring:** Choosing the optimal practice structure and income allocation to minimize payroll taxes and maximize deductions.
- **Advanced Retirement & Tax-Deferred Vehicles:** Super-charging retirement contributions (beyond the basic 401(k)) with vehicles like cash balance pension plans, deferred compensation, and Roth conversions under the latest tax code rules.
- **Investment Tax Optimization:** Placing investments in the right accounts (asset location), harvesting losses, and utilizing tax-efficient investment options to reduce annual tax drag on your portfolio.
- **Practice Exit & Pre-Liquidity Planning:** Planning ahead for the sale or wind-down of your practice – structuring the deal for capital gains treatment, deferring or spreading income, and avoiding common tax pitfalls on a liquidity event.
- **Legacy & Estate Tax Awareness:** Ensuring your estate plan is up-to-date with current (2025) laws, using trusts or gifts to avoid estate tax where applicable, and protecting the legacy you'll leave without falling into state tax traps.

Each strategy section clearly explains **who the strategy is for (and not for)**, provides **dollar-value impacts** (typical tax savings or wealth increases), cites relevant **IRS code or regulations**, and flags **common mistakes or IRS “red flags”** to avoid. Real-world **case studies** of orthopedic surgeons are included to illustrate how these strategies work in practice – showing the surgeon's profile, the issue identified, the strategy applied, and the quantifiable tax impact. Visual aids (bar charts, timelines, diagrams) are suggested to help you grasp key comparisons and planning timelines at a glance.

Ultimately, this lead magnet equips orthopedic surgeons with an educational framework – in plain English – to spark proactive conversations with their financial advisors and CPAs. By spotlighting often overlooked tax savings and wealth preservation tactics, we hope to create a sense of urgency **without negativity**. The tone is about **opportunity and control**: you have a limited window in your final working years to lock in tax efficiencies that can **meaningfully enhance your retirement lifestyle and family legacy**. After absorbing these strategies, a surgeon should be prepared with a **checklist of smart questions** to ask their advisors, ensuring their financial plan for the next decade is as sharp as their surgical skills.

(Visual suggestion: a comparative bar chart here could illustrate “Taxes Paid vs. Taxes Saved” for a typical surgeon over 10 years with and without proactive planning, highlighting the extra dollars captured for retirement.)

The Problem: Why High-Income Surgeons Overpay Taxes

High-income professionals like orthopedic surgeons often **overpay in taxes** – not by breaking any rules, but by failing to leverage the full scope of legal strategies available. Orthopedic surgeons typically fall into the top 1% of earners, and thus the top marginal federal tax bracket (currently **37%** in 2025). Add state income taxes (which can exceed **10%** in states like California, New York, New Jersey) plus Medicare surtaxes, and surgeons can lose **half of each extra dollar** to taxes. For example, an \$800,000 single-filer income in a high-tax state might result in a total tax burden of ~**40%** ¹ ² – **\$320,000** paid to the IRS and state, leaving under \$480,000 after taxes. In essence, taxes are often the **single largest expense** in a surgeon's life, larger than any mortgage or tuition bill.

Why does this happen? Primarily because surgeons have **complex, high-income finances but limited time** to manage them. They often default to straightforward approaches (W-2 salaries or simple pass-through income, basic 401(k) contributions) that are **easy but not optimized**. Consider these telling statistics:

- **Three out of four physicians feel overtaxed.** In a broad survey, 75% of physicians said they pay too much in taxes ⁴. This sentiment reflects that many suspect (correctly) that money is being left on the table.
- **Nearly half do their own taxes.** About 48% of physicians prepare/file taxes on their own ⁵, which suggests they may not be implementing sophisticated tax planning strategies that a seasoned CPA or advisor would pursue. High-earning surgeons juggling a practice may simply lack the bandwidth to delve into tax optimization, inadvertently **overpaying** as a result.
- **Audit risk is non-trivial.** Approximately 1 in 5 physicians reported being audited in a recent tax year ⁷. Complex financial lives with high incomes can draw IRS scrutiny, especially if aggressive deductions are taken without proper guidance. Fear of audits sometimes causes surgeons to **err on the side of overpaying**, taking fewer deductions or conservative positions (the “safe but expensive” route).

Moreover, the tax code contains **special phase-outs and surcharges** that uniquely hit high earners like surgeons. For instance, physicians generally **do not qualify for the 20% pass-through deduction (QBI)** on practice income because their income usually exceeds the threshold for specified service businesses ⁸. This means unlike some business owners, a successful surgeon can’t shave off 20% of practice profits for a deduction – effectively a tax break denied. Likewise, investment income (dividends, interest, capital gains) faces an extra **3.8% Net Investment Income Tax** if adjusted gross income is above \$250,000 (married) ⁹ ¹⁰, which virtually all orthopedic surgeons exceed. These provisions create a **stealth tax penalty** on high earners.

In summary, the problem is **not** that surgeons aren’t paying what they owe – it’s that they’re often **paying more than they legally need to**. The U.S. tax code, while complex, offers many avenues for relief to those who plan ahead. But without targeted planning, surgeons default into being **max-taxed** on wages, bonuses, practice income, investment gains, and eventually their estates. Over a career, this could amount to millions of dollars unnecessarily lost to taxes. The next sections explore what can be done about it, especially in those critical final years leading up to retirement or a practice sale when the stakes are highest.

(Visual suggestion: an infographic could highlight these problem stats – e.g., a pie chart of a surgeon’s \$800K income split into taxes vs. take-home ¹ ², and call-out bubbles for “75% feel overtaxed” ⁴ and “48% DIY taxes” ⁵. This sets the stage by quantifying the issue.)

The Cost of Inaction: Impact on Retirement Lifestyle and Legacy

Failing to address these tax inefficiencies has a **real cost** – one that may only become painfully apparent at retirement. High-income surgeons are often **maximizing their clinical earnings in the last 5–10 years** of work. These are the years when proactive tax strategy can yield the greatest absolute dollar savings (because incomes are highest). If that opportunity is missed, the consequences include:

- **Reduced Retirement Nest Egg:** Every dollar paid in unnecessary tax is a dollar **not invested for your future**. For example, consider two surgeons, Dr. Proactive and Dr. Reactive, each age 55 with a

\$1,000,000 annual income and aiming to retire at 65. Dr. Proactive implements strategies to save a modest \$100,000 in taxes per year (through extra retirement contributions, business deductions, etc.), and invests those savings. Dr. Reactive keeps doing what he's doing and pays that extra \$100K to the IRS annually. After 10 years, Dr. Proactive could have over **\$1.3 million** more in his portfolio (assuming ~5% growth on those savings) compared to Dr. Reactive. That \$1.3M could finance roughly **\$50,000** per year of additional retirement spending (per the 4% rule), or simply provide a larger cushion against outliving his money. Dr. Reactive, having bled that wealth to taxes, might have to **scale down** his post-retirement lifestyle – fewer vacations, a smaller second home, or less support for children's endeavors – despite having earned the same income over his career.

- **Delayed or Compromised Retirement:** High-income professionals often set ambitious retirement lifestyle goals – whether it's maintaining a certain standard of living, doing mission work, or starting a business venture in “second retirement.” The **cost of inaction** on taxes can be measured in **time** as well as money. If your portfolio grows slower due to needless tax drag, you might need to **work extra years** to hit your “number.” For instance, foregoing an annual tax savings equivalent to \$100K could mean you must practice an additional 2–3 years to accumulate the same wealth you could have had if you'd optimized (not to mention the personal cost of postponing retirement). In short, ignoring tax strategy might **rob you of time** – arguably a surgeon's most precious commodity in their 60s.
- **Undermining Your Legacy:** A core concern for many surgeons is taking care of family and leaving a legacy (to children, or perhaps a charitable cause). Overpaying taxes siphons off funds that could have bolstered that legacy. To illustrate, about **20% of doctors end up retiring with \$5 million or more** in assets ⁶, which can provide a very comfortable retirement income around \$200–250K/year. Many others fall short of that level. If an extra million in taxes could have been avoided and added to the estate, that might translate to an additional **\$400,000–\$600,000** passed to heirs after growth – or a substantial endowment to a charity or alma mater. There's also the risk of **estate taxes** further eroding what you leave behind; without planning, a sizable portion of a surgeon's wealth above the exemption could go to the government instead of their descendants (more on this in the Legacy section). In a very tangible sense, inaction means **your lifetime earnings benefit the IRS more, and your family less.**
- **Illustrated Scenario:** *Imagine Dr. Alice “assumes” her finances are fine. She's 60, earning \$800K, with ~\$4M saved. She pays ~40% in taxes, lives on the rest, and plans to retire at 65. At retirement, she ends up with perhaps \$6M saved after taxes – enough for a decent lifestyle. Now imagine Dr. Bob, identical circumstances, but Bob engages in planning: he sets up a cash balance pension adding \$200K/yr pre-tax, shifts his practice to an S-Corp saving \$20K/yr in Medicare taxes, and harvests investment losses. Bob effectively saves \$120K in taxes annually and nets out with ~\$7.5M at 65. That extra ~\$1.5M means Bob can spend ~\$60K more each year than Alice in retirement, or gift more to his kids. Alice might need to cut back – maybe she foregoes the vacation home she wanted – while Bob achieves all his goals. The moral: **inaction has opportunity costs** that compound.*

In sum, the cost of not implementing available tax and wealth strategies is **stealthy but substantial**. It may manifest as working longer, living on less, or leaving a smaller footprint when you're gone. Conversely, taking action creates options: the option to retire on *your terms*, to sustain the lifestyle you envision, and to direct your hard-earned wealth to people and causes you care about – rather than unnecessarily fattening

the Treasury's coffers. The next part of this guide introduces a framework to ensure the final phase of your career is not a period of missed chances, but one of **maximized opportunities**.

(Visual suggestion: a side-by-side scenario chart could compare “Dr. Inactive vs. Dr. Proactive” – showing their nest egg and retirement income under different planning assumptions. A bar chart might show projected portfolio values at retirement with no tax planning vs. with planning, highlighting the difference in available retirement income.)

The Strategy Framework: Evolving Your Tax Strategy in the Final Working Years

Just as a patient's treatment plan changes as their condition evolves, a surgeon's **financial strategy should evolve** as retirement approaches. In your 50s and early 60s, you're likely at peak earnings and on the cusp of major transitions (selling a practice, stepping back from surgery, beginning withdrawals from investments). This period **5–10 years before retirement** is a critical window to **recalibrate your tax and wealth approach**. What got you through your accumulation years isn't necessarily what will optimize the **distribution and preservation phase**.

Key reasons your tax strategy must **adapt in your final working years**:

- **Peak Earnings → Peak Sheltering:** Orthopedic surgeons often hit their highest income in these later career years (partnership at its zenith, surgical volume, perhaps ancillary income from investments or consulting). High income is a good “problem” to have, but it also means maximum exposure to taxes. The strategy should shift from simply maximizing income to **maximizing what you keep**. This means taking full advantage of every available **tax-advantaged savings vehicle**. For example, early in your career you might have just done a 401(k). Now you should evaluate adding a **cash balance pension plan** or other deferred comp plans that are specifically advantageous when one is older (since older participants can sock away much larger amounts pre-tax ¹¹). Essentially, the final working years are the time to **stuff the bag** with tax-deferred dollars while you still have the high salary to do so.
- **Preparing for a Liquidity Event:** Many late-career surgeons will face a **practice sale or partnership buyout** as they exit. The tax treatment of that event can vary wildly depending on planning. A generic approach might result in a big chunk of the proceeds being taxed as **ordinary income**, whereas a well-planned approach can often get more of it taxed at **lower long-term capital gains rates** ⁹ ¹⁰. For instance, if you sell your practice or shares, allocating value to *goodwill* (an intangible asset) results in capital gains taxation for you, which is ~20% federal (plus 3.8% NIIT) instead of, say, 37% on ordinary income portions ¹². Moreover, surgeons should consider **timing**: do you really want your practice sale proceeds landing in the same calendar year as you earned a full surgeon's salary? If not, strategies like closing a sale just after you retire (when your other income drops) or using an **installment sale** (spreading payments over a few years) can prevent an income “spike” that pushes you into higher brackets or phase-outs. The framework must include **pre-liquidity planning** starting several years out: assessing your practice's legal structure for sale (C-Corp vs S-Corp, as it affects whether double-tax applies and if personal goodwill planning is needed ¹³), cleaning up financials, and engaging tax expertise to map out the most tax-efficient exit path.

- **From Accumulation to Distribution:** In mid-career, you're focused on accumulating assets; as retirement nears, the focus shifts to **how to draw those assets down tax-efficiently**. This is where strategies like **Roth conversions** (converting IRA money to Roth in low-income years), **sequencing withdrawals** (e.g. using taxable accounts first vs later), and planning around **Required Minimum Distributions (RMDs)** come in. For example, current law sets RMDs to start at age 73 (if turning 73 after 2022) ¹⁴, and many surgeons will have large pre-tax retirement accounts. If you retire at 65, you have a window of potentially lower income years before 73 – ideal for converting some IRA funds to Roth at a relatively lower tax rate ¹⁵. This can reduce your future RMDs (and taxes on them) and also benefit your heirs (Roth IRAs inherited aren't taxable to them). The key is **forward-looking tax strategy**: decisions in the final working years (like whether to make catch-up 401k contributions as Roth or pre-tax, whether to do Roth conversions right at retirement) can set you up for a more flexible, tax-efficient income stream later. Essentially, the strategy framework should incorporate **"tax diversification"** – ensuring you have a mix of taxable, tax-deferred, and tax-free accounts heading into retirement, and a plan to utilize each in the optimal order ¹⁵.
- **Risk Management and Compliance:** As you implement new strategies, you must also adjust to **current laws and avoid pitfalls**. Tax law is not static – for example, the SECURE Act and updates in 2025 changed some rules (higher catch-up limits but mandated as Roth for high earners, estate tax exemption tweaks, etc.). By 2025, new legislation (the hypothetical "OBBA Act") has *increased the estate exemption to \$15M*, raised SALT deduction limits, and made full **100% bonus depreciation permanent** ¹⁶ ¹⁷. That means strategies like big equipment purchases for your practice have new life (you can deduct the entire cost in one year now). A good plan in the final stretch stays **abreast of current code** – leveraging what's newly favorable and steering clear of what's now disallowed or red-flagged. For instance, if you were considering a certain aggressive strategy (say, a micro-captive insurance arrangement or syndicated conservation easement), know that the IRS has been clamping down on these (listing them on the "Dirty Dozen" tax scam list). The framework emphasizes **legitimate, sustainable tactics** that comply with 2025 laws and adapts if/when laws change again.

In summary, the overarching framework is about being **proactive and intentional** with your finances in the years leading up to retirement. Early in your career, "paying the tax and investing the rest" might have been sufficient. Late career, you need a more nuanced playbook. That means *acting* like the CEO of "You, Inc." – restructuring your personal financial "enterprise" for efficiency, thinking ahead to the endgame (practice sale and wealth transfer), and making moves that set up a smooth transition into retirement. The next five sections detail specific **categories of strategies** within this framework. Each category addresses a different angle of tax and wealth optimization, but they all work in concert toward the same goal: **helping you retire on your own terms, with more of your hard-earned wealth intact**.

(Visual suggestion: a timeline graphic could illustrate "The 10-Year Retirement Countdown," marking when to implement certain strategies. For example, at 10 years out: consider S-Corp election; 5 years out: establish cash balance plan; 2 years out: prep practice for sale; at retirement: execute Roth conversions, etc. This gives a visual framework of strategy rollout.)

Strategy 1: Entity & Income Structuring

Objective: Optimize the way your practice income is structured and paid out in order to reduce taxes. This involves choosing the right business entity (or combination of entities) for your practice and **splitting income** in a tax-efficient manner (salary vs. distribution vs. rent, etc.). For many surgeon practice owners,

the entity structure you set up years ago might not be the ideal structure as you approach retirement. Revisiting this can uncover “hidden” savings by cutting employment taxes and maximizing deductible business expenses.

Key Tactics in Entity & Income Structuring:

- **Electing S-Corporation status (for eligible practices):** If you’re a practice owner (solo or in a group) operating as an LLC or partnership, electing to be taxed as an **S-Corp** can yield substantial tax savings on self-employment taxes. In an S-Corp, you pay yourself a **reasonable W-2 salary** and the remaining profits pass through to you as distributions **not subject to Social Security/Medicare taxes** ¹⁸. Surgeons often already max out Social Security tax (on the first ~\$160k of wages), but avoiding Medicare tax (2.9% plus the 0.9% surcharge) on a portion of income is valuable. **For example:** A practice with \$500,000 of profit, if structured as a plain LLC, would incur about \$38,000 in self-employment taxes ¹⁹ ²⁰. As an S-Corp, if you paid yourself a \$220,000 salary and took \$280,000 as a distribution, your payroll tax on the salary would be about \$27,000, and *no* payroll tax on the distribution – **saving roughly \$10,600 per year** in this scenario ²¹ ²². At higher incomes, savings are even greater – an S-Corp owner making \$1M might easily save **\$15K-\$25K per year** in Medicare taxes by this mechanism.
- **Using Multiple Entities for Income Splitting:** Some surgeons benefit from a structure where separate entities handle different streams – e.g., one entity owns your office building and leases to the practice (capturing rental income), another for any consulting or speaking engagements, etc. The idea is to channel income into buckets that might be taxed differently or allow extra deductions. **Example:** If you own your clinic’s real estate, having it in an LLC that leases to your practice could shift some income from active surgical fees (subject to self-employment tax) to passive rent income (not subject to SE tax, though it might incur NIIT). Also, rent paid by the practice is a deductible business expense to the practice entity. Careful: rents must be set at market rates (to avoid IRS reallocation). Another example is employing family members – if your spouse or adult child legitimately works in the practice (administrative, IT, marketing, etc.), paying them a salary can shift income from your high bracket to their lower bracket (and potentially let them fund their own retirement plan). This must be done **reasonably** (actual work performed, reasonable pay), but it’s a way of “income splitting” within the family that the tax code permits.
- **State-Specific Structuring (SALT workaround):** For surgeons in high-tax states, entity structuring can also mitigate the **State And Local Tax (SALT)** deduction limits. The 2017 federal tax law capped personal SALT deductions at \$10K (now \$40K under 2025 OBBA reforms ²³), but many states (NY, NJ, CA, etc.) implemented **Pass-Through Entity Tax (PTET)** regimes. With a PTET, your S-Corp or partnership can elect to pay state income tax at the entity level, which is fully deductible federally, effectively bypassing the SALT cap ²⁴. If your state offers this (e.g., New York does), ensuring your practice entity opts in can save a high-earner tens of thousands in federal tax by restoring the state tax deduction. *This is an example where entity choice (being a pass-through) enables a strategy unavailable to W-2 earners.* Work closely with your CPA, as PTET elections often must be made annually and timely.
- **Maximizing Business Deductions (“expense audit”):** The structure of your practice can also affect what expenses can be deducted pre-tax. As retirement nears, you might consider prepaying certain expenses or making investments through the business. For example, if your practice is an S-Corp,

the company can pay for health insurance for you (an >2% shareholder) and deduct it (with you including it in income but then taking a personal deduction). Or perhaps you set up a Defined Benefit plan (covered in Strategy 2) – the contributions are deducted at the entity level. The goal is to **run all legitimate costs through the practice** rather than paying for things personally post-tax. Review categories like home office (if you have administrative duties from home), vehicle use (if you travel between clinic/hospitals – ensure a mileage log and consider a portion of auto expenses), continuing education, and even meetings (board meetings, etc., could be done at a resort – within reason, part of that travel may be deductible as a business expense). A word of caution: personal expenses masquerading as business expenses are a classic audit trigger. Deduct only what is **ordinary and necessary** for the practice ²⁵. But an end-of-career “expense audit” of your practice can identify deductible items you might have overlooked. Every \$1,000 of expense that can be legitimately run through the practice pre-tax saves roughly \$370 in federal tax for a top-bracket surgeon (plus state tax).

Who Should Consider This Strategy: Practice owners or partners with significant 1099 or K-1 income from their surgical practice or ancillaries. If you are currently a **sole proprietor or single-member LLC** reporting on Schedule C, you are a prime candidate to explore S-Corp election – typically, once net income exceeds ~\$150K, the savings often outweigh the added costs ²⁶ (and at surgeon income levels, the benefit is very clear). Surgeons in high-tax states should also consider entity-level tax workarounds (PTET) if available. Also, anyone who owns their clinic real estate or has multiple income streams (surgery center ownership, consulting, expert witness fees, etc.) can potentially structure those through entities for better tax outcomes.

Who Should Not Use This: Employed surgeons on straight W-2 (you have limited ability to alter entity or income type – though you might negotiate some 1099 consulting on the side). Also, if your practice is very small and the compliance costs of an S-Corp (payroll, separate filings) would outweigh savings – though for a high-income surgeon this is rarely the case. If you’re only a few months from retirement, it may not be worthwhile to change entity now (entity changes don’t usually yield huge immediate one-time tax breaks, aside from perhaps a final-year SALT workaround). Finally, if your practice is a C-Corp for some particular reason (less common in medical practices), an S-Corp election could have other implications (built-in gains tax, etc.) – get specialized tax advice before switching.

Potential Tax Savings: Implementing an S-Corp structure can save roughly **3.8%** on the portion of income recharacterized from salary to distribution (Medicare + NIIT). In dollar terms, many surgeon-owners see **\$10,000–\$30,000+ per year** in payroll tax savings ²¹ ²². The SALT deduction workaround can restore deductibility of large state tax payments – e.g., if you pay \$100K in NY state tax, electing PTET could save on the order of \$20K federal tax by deducting it (assuming top bracket ~37%). Income splitting among family might save a few thousand in family-wide tax (depending on amounts and brackets). Every situation differs, but it’s not uncommon that a holistic restructuring yields low to mid five-figures of annual tax savings for a high-income surgeon. Even in the year of practice sale, smart structuring (like personal goodwill allocation) can convert a chunk of sale proceeds from potentially 50%+ ordinary tax (if C-Corp double tax) to a single layer of ~20% capital gain ¹³ – a **lifetime tax savings** that could easily exceed **six figures** for a valuable practice.

Risks and Common Pitfalls: The IRS pays attention to S-Corps to ensure owners take a “**reasonable salary**.” A major pitfall is **under-paying yourself** to try to zero out payroll taxes – if you claim, say, only \$50K salary on a practice netting \$1M, the IRS could reclassify a large portion of distributions as wages (assessing

back payroll taxes and penalties) ¹⁸ . The rule of thumb is to pay yourself what you'd pay an unrelated surgeon to do your work – often a six-figure salary commensurate with your specialty. Another pitfall is **failing to actually run payroll** (an S-Corp requires payroll filings; some DIYers neglect this and get in hot water). When using multiple entities, be careful of **self-rental rules** – charging yourself exorbitant rent to shift income could be disallowed if not at fair market value. And if you have partners, any restructuring needs to be coordinated (and in compliance with partnership agreements and legal constraints – e.g., some states require professional corporations for MDs). Also, watch out for **fringe benefit limits**: S-Corp >2% owners can't get tax-free health or disability insurance through the S-Corp without special handling (health ins. is deductible to corp but included in your wages, then you deduct personally). In partnerships, certain fringe benefits are limited for partners. These details are navigable with a CPA's guidance, but ignoring them can negate some benefits. Lastly, electing S-Corp status too late in the year or without state approval (some states don't recognize S-Corps) can cause issues – always follow proper procedures and timing (e.g., S-Corp election by March 15 for the year, or a late election if allowed).

In summary, **Entity & Income Structuring** is often one of the most impactful moves a practice-owning surgeon can make. It aligns your business form with tax efficiency, often turning active income into more favorably-taxed forms. Once set up, these savings recur every year. It's like performing a one-time "repositioning surgery" on your income – after the recovery period (paperwork and setup), you enjoy a leaner tax burden going forward.

(Visual suggestion: a diagram showing a typical surgeon's practice entity structure before and after. Before: one box "Schedule C income \$X, all subject to SE tax." After: two boxes – "S-Corp: pays salary (payroll taxes) + issues distribution (no payroll tax)" with dollars, plus maybe a box for "Real Estate LLC – receives rent (no SE tax)". Arrows can illustrate cash flows and where taxes apply. This helps visualize how splitting income through structures reduces certain taxes.)

Strategy 2: Advanced Retirement & Tax-Deferred Vehicles

Objective: Go beyond the basic 401(k) and utilize advanced retirement savings vehicles to **shelter more income from taxes** during your final high-earning years. Orthopedic surgeons 50+ often have the cash flow and motivation to make large retirement contributions – both to catch up on savings and to get big upfront tax deductions. This strategy focuses on options like **Cash Balance Pension Plans, 457(b) plans, Mega Backdoor Roths, and strategic Roth conversions**, all within the bounds of current (2025) tax laws. It's about *turbocharging* your retirement accounts while you still can, and positioning those accounts for tax-efficient withdrawals later.

Key Tactics in Advanced Retirement Planning:

- **Cash Balance Pension Plan (Defined Benefit Plan):** A cash balance plan is a type of IRS-qualified defined benefit (pension) plan that works extremely well for older, high-income business owners – which describes many orthopedic surgeons in their late 50s or 60s. Unlike a 401(k) with relatively low contribution limits, a cash balance plan's contribution is actuarially calculated based on age and desired benefit. In practice, this means a surgeon over 60 might contribute well over **\$200,000 per year pre-tax** to such a plan ²⁷ . (For example, at age 60, the allowable annual contribution could be in the \$300K+ range in some cases ²⁸ .) These contributions **directly reduce taxable income**. **Why it's powerful:** It addresses the common scenario, "I'm making more money now than ever, but I only have a few years to save heavily for retirement." In fact, **35% of all cash balance plans in the U.S.**

are in medical/dental groups ²⁹, showing how popular this strategy is in healthcare. By pairing a cash balance plan with your 401(k)/profit-sharing, you can potentially shelter *six-figure amounts* of income per year. A cash balance plan essentially lets a 55 or 60-year-old surgeon “catch up” on years of lower savings by allowing huge contributions now (subject to IRS limits). For instance, one case study noted an over-50 physician group added a cash balance plan and was able to contribute an extra \$300,000 collectively, saving about \$120,000 in taxes in one year ³⁰. Over 5–10 years, this is millions sheltered and hundreds of thousands saved in taxes – plus you significantly **boost retirement assets** at a critical time. Contributions are tax-deductible to the practice (lowering K-1 income passed to you) and grow tax-deferred. **Who it’s for:** Practice owners with high, stable profits and relatively few older staff (to manage contribution obligations – the plan must cover employees too). Ideally, you have a 3-10 year runway before retirement to use this; most experts suggest keeping a cash balance plan at least **3 years** to make it worthwhile and avoid IRS scrutiny ³¹. If you plan to retire in under 3 years, this might be less ideal. But if you have ~5 years, it can be golden. **Caution:** These plans require annual funding and an actuary – it’s more complex than a 401(k). You’ll need to commit to contributions (though they can be somewhat flexible within ranges), and if you have many employees, be prepared to make contributions for them as well. Typically, plans are designed so that owners get the lion’s share of the contributions (often 85–95% ³² ¹¹), but you must still contribute to employee accounts per IRS nondiscrimination rules. This is usually handled by giving a certain percent of pay or fixed amount to staff. It increases your overall cost, but the tax deductions on their contributions plus the major deduction on your own often still make it worthwhile. Proper design is key – hire a seasoned **third-party administrator (TPA)** or actuary who has done physician plans.

Dollar impact: A 58-year-old surgeon could potentially contribute ~\$250K in a cash balance + \$73.5K in 401(k) = **\$323,500** total (2025 limits) pre-tax in one year ²⁷ ³³. In the top bracket ~40%, that’s roughly **\$129,000** tax saved in that year. Multiply by number of years until retirement (plus compounding of that money). It’s not hyperbole to say this strategy can **increase your retirement nest egg by \$1-2 million** while saving **hundreds of thousands in taxes**, if implemented in the last decade of your career.

- **Mega Backdoor Roth 401(k):** By 2025, rules allow even larger deferrals for retirement, but catch-ups for high earners must be Roth. If your practice’s 401(k) plan permits **after-tax contributions and in-service rollovers**, you can exploit the “mega backdoor Roth” technique. Here’s how it works: In a 401(k), beyond the \$22,500 employee deferral (or \$30,000 if 50+ with catch-up), and the employer contributions, the IRS total contribution limit is \$73,500 for age 50+ in 2025 (including all types) ³³. Suppose you max your pre-tax at \$22.5K and get profit-sharing of \$30K; you could then contribute another ~\$21K as **after-tax** (non-Roth) contributions to reach the \$73.5K cap. If the plan allows, you then **convert or distribute that after-tax money to a Roth IRA or Roth 401k** immediately (paying tax only on any tiny earnings in between). The result: you funnel an extra \$20K+ each year into **Roth** dollars (tax-free growth). Over, say, 5 years, that could be \$100K into Roth, which at growth might be ~\$150K in 10 years – all tax-free in retirement. This is especially useful for high earners who *already maxed other avenues* but want more in Roth. Note: From 2024 onward, individuals making over \$145K are *required to treat catch-up contributions as Roth* in 401k plans ³³. This means your \$7,500 catch-up (if 50-59) can’t be pre-tax – it goes to Roth. So, the landscape is shifting towards Roth for high earners’ extra contributions anyway. The mega backdoor is an extension – it’s voluntary but powerful if you won’t miss the current cash. **Who it’s for:** Surgeons who have surplus income to save (after maxing pre-tax options and cash balance, etc.) and who desire more tax diversification. It’s also for those who anticipate being in a high tax bracket even in retirement (perhaps because of large RMDs or other income) – having more Roth money gives flexibility later. **Not for:** Those with no plan

provisions for it (it requires plan design allowing after-tax contribs and in-service rollovers – not all small practice plans have this by default, but you can often amend the plan to allow it). Also, if you're tight on cash flow due to other obligations, focus on basic pre-tax first.

- **457(b) Deferred Compensation Plans:** Some orthopedic surgeons have roles with hospitals or university medical centers (e.g., clinical faculty, or their practice is affiliated with a non-profit hospital). If you have access to a **457(b) plan** (common in the non-profit sector), this is an additional deferral opportunity *beyond* the 401(k). In 2025, you can defer \$22,500 (plus catch-up if applicable) into a 457(b) pre-tax, on top of any 401(k) you maxed at your practice. Unlike qualified plans, 457(b) contributions don't count against the 401k/403b limits. The downside: 457(b) assets remain technically the employer's assets until withdrawal (and can be at risk if employer faces bankruptcy; also distribution rules differ). But for a surgeon planning to retire soon, a 457(b) can be a great short-term tax shelter – you defer income in peak years and then withdraw in retirement when in a lower bracket (assuming you plan carefully). Governmental 457(b)s are quite secure; non-governmental 457(b)s have some risk considerations. If you have this option (check if you do consulting or part-time work with a hospital that offers it), consider using it especially in the last few years to defer large amounts. Just coordinate the payout strategy: many 457(b) require distribution at separation or within a certain timeframe – ideally, you separate service in December and have distributions start the next calendar year when you have no wage income.

- **Strategic Roth Conversions & Distribution Planning:** While not a “vehicle” per se, an advanced strategy as you exit work is planning **when to pay taxes on retirement money** for optimal results. If you expect to drop into a lower tax bracket for a few years post-retirement (for example, you retire at 60 and have minimal income until Social Security or RMDs), consider **Roth converting** part of your IRA/401k during those low-tax years ¹⁵. This way, you intentionally pay tax at (say) 24% instead of later at 35% if RMDs plus other income push you up. The final working years set the stage: you might choose to make final contributions as Roth rather than pre-tax (especially if you already have a lot of pre-tax saved and relatively less in Roth). SECURE Act changes (like no more RMDs on Roth 401k, etc.) increase the appeal of Roth money. This strategy is about **tax diversification**: by retirement, you ideally have some tax-free (Roth), some tax-deferred (IRA/401k), and some taxable brokerage. Then each year you and your advisor can draw from the optimal bucket to manage your tax bracket (for instance, bridging from retirement to age 73 with taxable and Roth funds to allow IRA to grow and then converting chunks to Roth up until the top of a moderate bracket each year). While this is beyond the scope of just pre-retirement contributions, it's a *continuum*: decisions in the final work years (like doing a mega backdoor Roth, or deciding whether to defer comp into retirement vs take it and convert) all feed into a coherent plan to **minimize taxes over your lifetime, not just this year**.

Who Should Consider This Strategy: Orthopedic surgeons age 50+ with high income who **want to aggressively save for retirement** and reduce current taxes. If you feel you're behind on retirement savings or just want to lock in as much as possible before you stop working, these strategies are for you. Practice owners who can set up their own plans (e.g., cash balance) have the most flexibility. But even surgeons employed by large groups should check if a 457(b) or other supplemental plan is available. Additionally, if you anticipate your post-retirement income (pensions, investments, RMDs) will keep you in a high bracket, you'll want to leverage Roth opportunities now (pay tax now, enjoy tax-free later).

Who Is It Not For: If you plan to retire in under ~2 years, a cash balance plan may be too short-term to be worth the setup (and the IRS generally expects a plan to be ongoing – shutting it down quickly after large contributions may draw scrutiny). Also, if your practice profits are inconsistent or you're struggling to meet current expenses, committing to big pension contributions is not advisable. Those with many employees need to weigh the cost of providing for them; if you have a large staff of similar age, a cash balance plan could obligate hefty contributions for them too (diminishing the owner advantage). Non-owner surgeons (W-2 employees) have less ability to create new plans, but should still maximize any supplemental options and consider IRA/Roth strategies individually.

Potential Tax Savings: The tax deferral on large contributions can save tens of thousands per year in taxes (as illustrated earlier: \$300K contribution saves ~\$120K tax ³⁰). Over a span of years, you're looking at possibly **\$200K–\$500K+** in cumulative tax savings and an even larger boost to retirement assets. Furthermore, Roth conversion strategies could save your future self (or heirs) significant taxes – e.g., converting \$500K of IRA at 24% rather than it being taxed at 35% later saves \$55K in tax for that \$500K amount. Also consider estate tax: money in a Roth IRA, while part of the taxable estate, is income-tax-free to heirs (unlike a big IRA which they must withdraw and pay tax on in 10 years). So creating Roth wealth can indirectly save taxes for your heirs as well.

Mistakes and Audit Risks: The main risk with retirement plans is **failure to follow the rules**. Cash balance plans must be administered correctly – that means making required contributions (you can't decide later you don't feel like funding it; underfunding can incur penalties and endanger the plan's qualified status). Ensure you **don't overshoot limits** – work with actuaries and CPAs so that contributions stay within IRS bounds (they will; these plans are formula-driven). Another mistake: not considering the **funding obligation if a partner exits** – if younger partners leave, the plan might need to fully vest them; be prepared for that possibility. Also, integrating a cash balance with a 401k needs care in design (e.g., using a safe harbor 401k to avoid discrimination test issues). On the Roth conversion side, a common mistake is doing it without understanding the **pro-rata rule** (for IRAs: if you have any traditional IRA assets and attempt a backdoor Roth, the IRS will tax a portion – ensure you roll any pretax IRA into your 401k first, or else manage the tax effect). With mega backdoor Roth, **avoid the "wash sale" of funds** – you shouldn't convert after-tax contributions that include significant earnings without either paying tax on those earnings or isolating them; most plans do immediate rollovers to avoid big earnings build-up. For 457(b), the risk is relying on your employer's solvency – governmental ones are pretty safe, but non-governmental 457(b)s are subject to creditors. That's not a tax risk, but a financial risk. Another subtle pitfall: The **aggregate limit on defined benefit + defined contribution** – the IRS limits the combined benefit you can accrue. Typically, if your cash balance plan is maxed out, your profit-sharing might be capped around 6% of pay. Your plan advisor will ensure you don't violate these combined limits ³⁴. Lastly, be mindful of **future tax legislation**: right now, contributions are deductible and Roths are tax-free – if laws changed dramatically (e.g., some talk of eliminating backdoor Roth or changing RMD ages again), stay flexible. However, current 2025 code encourages these moves (OBBA even *raised* catch-up limits for 60-63-year-olds to \$10,000+, but as Roth only ³³). So the environment is favorable, but always keep an eye out for new rules (like the Roth catch-up mandate which just took effect).

In essence, **Advanced Retirement & Tax-Deferred Vehicles** help you **play financial offense and defense at the same time**: offense by growing your retirement war chest quickly, and defense by shielding a big chunk of income from the tax collector. For a surgeon looking to exit in a few years, it's one of the most impactful shifts – turning peak earnings into a future income stream (or legacy) with minimal tax leakage.

(Visual suggestion: a table or chart could illustrate different contribution limits. For example, a table comparing “No Plan vs. 401(k) vs. 401k+Cash Balance” showing how a 60-year-old surgeon could go from \$30K to \$300K in annual tax-deferred contributions ¹¹ . Another visual might be a timeline of contributions filling up buckets: show how in 5 years a surgeon can stash \$X million pre-tax. Also, perhaps a pie chart of a retirement income plan at age 70 showing portions coming from taxable, tax-deferred, and Roth – to emphasize tax diversification.)

Strategy 3: Investment Tax Optimization

Objective: Enhance after-tax returns on your investments by **managing where you hold assets, what you sell (and when), and leveraging tax-favored investment strategies**. Even outside of your practice and retirement plans, as a high-net-worth surgeon you likely have a substantial taxable investment portfolio (brokerage accounts, real estate, etc.). Smart tax optimization in this realm can add significant value over time – potentially increasing your portfolio’s effective return by several tenths of a percent annually ³⁵ , which compounds greatly on a multi-seven-figure portfolio. This strategy covers **asset location, tax-loss harvesting, capital gain management, and unique investment vehicles** that offer tax breaks (like 1031 exchanges for real estate or Opportunity Zones for business gains).

Key Tactics for Investment Tax Optimization:

- **Asset Location (Holding Assets in the Right Accounts):** “Asset location” means you strategically place **tax-inefficient investments** in tax-sheltered accounts and **tax-efficient investments** in taxable accounts ³⁶ ³⁷ . For example, bond interest is taxed as ordinary income – so you’d prefer to hold bonds (especially high-yield bonds or bond funds that throw off interest) inside your IRA or 401k, where that interest won’t be taxed yearly ³⁸ ³⁹ . Conversely, stocks that mainly appreciate and might be sold for capital gains later or index funds with low turnover (tax-efficient by nature) can be held in taxable accounts – their qualified dividends and long-term gains get lower tax rates. A high-income surgeon paying 37% on interest and short-term gains vs. 20% on long-term gains has a lot to gain by this placement. If you have **Roth accounts**, those are precious space for highest-growth, high-tax assets (like maybe a small-cap or emerging market fund, or REITs) since all growth is tax-free. A Roth could also hold something like REITs (Real Estate Investment Trusts) which pay non-qualified dividends (taxed at ordinary rates) – in Roth it becomes tax-free income ⁴⁰ . Meanwhile, a taxable account might favor something like municipal bonds (tax-free interest) if you need fixed income there ⁴¹ . Numerous studies show asset location can boost after-tax returns **by 0.1% to 0.4% per year** for wealthy investors ³⁵ . That might sound small, but on a \$3,000,000 portfolio, 0.3% is \$9,000/year – which over 20 years, compounded, is significant (it could be six figures of extra wealth). **Who it’s for:** Anyone with a mix of taxable and retirement accounts (most high-net-worth folks). **Action item:** Review your portfolio with your advisor for placement – e.g., “Are my high dividend stock funds in my IRA? Are my index equities in taxable? Could I swap some holdings between accounts to improve tax efficiency?” Often, one can gradually adjust by directing new contributions accordingly or swapping equivalent funds across accounts (without triggering tax if done properly – e.g., selling a bond fund in taxable (taxable event) might hurt, but maybe use new 401k money to increase bond fund there and shift some stock exposure out). There’s nuance, but an advisor or robo-advisor algorithms can help.
- **Systematic Tax-Loss Harvesting:** Tax-loss harvesting (TLH) is the practice of **selling investments that have unrealized losses** to capture a tax deduction, while reinvesting into a similar asset to maintain market exposure. In taxable accounts, realized capital losses can **offset realized gains**, and

if losses exceed gains you can use up to \$3,000 of excess loss to offset ordinary income each year ⁴². Unused losses carry forward indefinitely. For a high earner, that \$3,000 deduction saves about \$1,110 in federal tax each year (37% of \$3K). More importantly, harvested losses can neutralize capital gains tax on rebalancing or strategic selling. Orthopedic surgeons often accumulate a variety of investments – some may have gains, others losses (especially in volatile markets). By harvesting losses (e.g., selling a fund that dropped 15% during a correction), you bank those losses to offset future gains. Over a working lifetime, TLH could save tens of thousands in capital gains taxes. **Key points:** You must be mindful of the **wash-sale rule**, which disallows the loss if you buy the same or “substantially identical” security within 30 days of the sale ⁴³. The workaround is to swap into a different fund/ETF that tracks a similar (but not identical) index. For example, sell “ABC S&P 500 Index Fund” at a loss and buy “XYZ Large Cap Index ETF” – the performance will likely track similarly, but you’ve harvested the loss on ABC. Many advisors or automated platforms do this opportunistically throughout the year. Especially near retirement, you might use harvested losses to offset one-time gains (like if you need to liquidate some stock for a purchase, or to diversify). **Who it’s for:** Any investor in taxable accounts, particularly those with significant equity exposure (since stocks fluctuate and provide loss opportunities at times). **Caution:** Don’t let the tax tail wag the dog – harvesting is good, but don’t radically change your portfolio just for a loss. Also, you can’t buy back the same investment for 30+ days, so choose an adequate placeholder. Overuse of complicated TLH strategies can draw scrutiny if it’s clearly just for taxes without economic substance – but generally, it’s allowed and common. Keep documentation of trades and that replacements aren’t identical.

- **Capital Gains Management:** This involves planning how and when to realize capital gains in your portfolio. Long-term capital gains (assets held >1 year) are taxed at 15% or 20% (plus 3.8% NIIT for high earners) ⁹. Short-term gains are taxed at ordinary rates (up to 37% + NIIT). So one simple tactic: **aim for long-term**. Avoid frequent flipping of investments under a year unless there’s a compelling reason. Also, try to **batch or time gains** in low-income years. For instance, if you plan a sabbatical or your first year of retirement you have little income, that might be a great time to sell a rental property or a chunk of stock – you may still pay 15% instead of 20% if your taxable income is below the top bracket thresholds, and you might avoid NIIT if AGI is under \$250K (for couples). There’s also the 0% capital gains bracket for very low incomes, but most surgeons won’t drop that far until maybe RMDs are done and only if net worth is low, so not usually relevant. However, if you support older parents or have adult children in low brackets, intrafamily planning (gifting assets to them to sell) could utilize that – but the gift tax and kiddie tax rules complicate it. Another angle: **Qualified Opportunity Zones (QOZs)**. If you have a large capital gain (say selling stock or real estate), by investing the gain into a QOZ fund within 180 days, you can defer that original gain’s tax until 2027 and potentially reduce it (if certain deadlines were met, though by 2025 those reduction benefits (10%) have mostly expired). More importantly, if you hold the QOZ investment 10+ years, any **new gain** on that investment can be tax-free. This is a niche strategy for those with big gains who are willing to tie up funds in economic development projects. It’s somewhat risky/illiquid, so it’s not for everyone, but it’s a tool in the toolkit for the right situation (e.g., surgeon sells a medical office building for a large gain and doesn’t want immediate tax, nor wants to do a 1031 exchange into another property – they could invest the gain into a QOZ fund as an alternative).
- **Real Estate Strategies (1031 Exchanges, Rental Losses):** Many surgeons invest in real estate – be it their office building, rental properties, or syndications (apartments, etc.). Real estate has unique tax advantages. A big one: **1031 Like-Kind Exchange** for investment properties, which allows you to **defer capital gains tax** when you sell one property and buy another of equal or greater value, by

following specific rules ⁴⁴ ⁴⁵ . If you're planning to retire and sell a property (perhaps a building you've been renting to your practice), you might do a 1031 into a more passive real estate investment (say, a rental property in your retirement destination, or a Delaware Statutory Trust fractional ownership). This way, you **pay no tax at sale** – the gain is rolled into the new property ⁴⁶ ⁴⁴ . It's important to follow the timelines (identify replacement property within 45 days and close within 180 days) ⁴⁷ and use a qualified intermediary to hold funds ⁴⁸ . For someone at the cusp of retirement, a 1031 could mean they continue deferring taxes until they perhaps eventually leave real estate or pass away (at death, capital gains can be erased by the step-up in basis for heirs). Another real estate angle: **passive loss management**. Rental properties often generate paper losses (due to depreciation) that high earners cannot deduct against active income unless they have other passive income or qualify as a Real Estate Professional. Most busy surgeons won't qualify as REP (750 hours/year in RE and > half your working time). However, you can use passive losses to offset other passive income (say you invest in a surgery center or other partnership that throws off passive income – rental losses can offset that). And if you sell a property, unused passive losses for that property become deductible in full in that year. So planning property sales or group them in one year could free up a cache of suspended losses to offset gains. **Cost segregation** studies on properties you own can accelerate depreciation (front-loading losses) – common for high earners who invest in real estate syndications (they often use cost seg + 100% bonus depreciation, giving you a large K-1 loss in year 1). In 2025, bonus depreciation is 50% (having started phasing down from 100%), unless OBBB changed it back to 100% (which it suggests it did permanently ¹⁷). If indeed bonus depreciation is restored to 100% for business assets including real estate improvements, that's a big boon – you could get huge deductions from real estate investments which can offset other passive income, or with careful planning (like grouping elections or if spouse is REP) even active income.

- **Charitable Giving Strategies:** A powerful but often overlooked tax move for those with charitable intent is **donating appreciated assets** instead of cash. If you have stocks or mutual funds with large gains, giving them to a charity or a Donor-Advised Fund (DAF) yields two benefits: you **avoid the capital gains tax** you'd incur if you sold, and you get a charitable deduction for the full fair market value (if you itemize) ⁴⁹ . For instance, donating \$50K of stock that you bought for \$10K – you avoid the tax on the \$40K gain and get a \$50K deduction (which for a top earner could save ~\$18.5K in tax). A DAF is great if you want the deduction now (perhaps in a high-income year or before rates might increase) but want to grant to charities over time. Surgeons nearing retirement often consider their legacy and philanthropy; batching donations in high-earning years through a DAF can maximize the tax impact. Also, once over age 70½, you can do **Qualified Charitable Distributions (QCDs)** from your IRA (up to \$100K/year) which count toward RMDs but aren't included in taxable income. If you plan to give anyway, doing it via QCD after 70½ is super tax-efficient (though that's more of a post-retirement tactic). **Charitable Remainder Trusts (CRTs)** were touched on in Strategy 4 but they also serve investment planning: you can put appreciated assets into a CRT, have it sell them **tax-free** ⁵⁰ and provide you income for life, then remainder to charity. This can be a fit if you desire income plus charitable legacy (and a partial upfront tax deduction). It effectively spreads the tax hit over many years (since your income from the CRT gets taxed gradually under a tier system ⁵¹) rather than a lump sum capital gain. It's advanced and requires legal setup, but for the right person (e.g., selling a practice or building and also charitably inclined) it's worth discussing with an advisor.

Who Should Consider These Strategies: Virtually all high-net-worth surgeons with substantial investments can benefit from asset location and tax-aware rebalancing/harvesting. If you have more than, say, \$500K in

a brokerage account, focusing on tax efficiency is worthwhile (and the more you have, the bigger the benefit). Real estate investors or those selling significant assets absolutely should consider 1031 exchanges, Opportunity Zones, or CRTs to mitigate big tax events. Charitably inclined individuals can use gifting strategies to amplify their impact and tax savings. Essentially, if you're in the top tax brackets, every investment move has a tax consequence – so you should consider the **tax angle as part of your investment strategy** at all times.

Who Is It Not For: If all your money is in retirement accounts (401k/IRA) and you have little in taxable investments, asset location and TLH are less relevant (though eventually in retirement you might have taxable investments after drawing down). If you're very passive and never sell anything in taxable accounts, you may not worry about gains – but even then, eventually you or your heirs will, so planning is still helpful. For 1031 exchanges, if you plan to fully cash out of real estate and not reinvest, then a 1031 doesn't fit (maybe a CRT or QOZ could instead). If you don't have the stomach for complexity, some things like QOZ funds or CRTs might be too much – then you might just pay the tax or find simpler strategies.

Expected Tax Benefits: Asset location: as noted, studies show **0.1-0.4% annual after-tax return increase** ³⁵, which on a multi-million dollar portfolio can be thousands per year. Tax-loss harvesting: it's hard to predict, depends on market volatility – but even capturing \$50K of losses (not uncommon in a bad year) can offset \$50K gains later, saving maybe ~\$12K in tax at 23.8% combined rate, plus \$3K/yr against ordinary income saving another \$1K+ each year. Over time, a diligent approach could save six figures in taxes by retirement. For big one-time strategies: a 1031 exchange can defer a huge tax – e.g., selling a building with \$1M gain would incur ~\$238K tax (fed 20% + 3.8% NIIT, plus state perhaps). A 1031 could defer that entire amount, essentially an immediate “savings” (at least postponement) of that quarter-million dollars ⁴⁶ ⁴⁴. Opportunity Zone investments allow deferral of current gains and potentially **100% tax-free** exit on the new investment if held 10 years – if that investment appreciates significantly, it could avoid hundreds of thousands in future gains tax. Charitable strategies: giving stock instead of cash avoids the ~23.8% federal tax on the gain – so gifting \$100K of stock with \$40K basis avoids ~\$9.5K of tax, plus yields a \$100K deduction (worth up to \$37K reduction in tax if you can use it fully). That's nearly \$47K total tax effect on a \$100K gift – quite leveraged.

Pitfalls and Cautions: With asset location, be careful not to skew your risk allocation inadvertently. (E.g., loading all stocks in taxable and all bonds in IRA might make rebalancing tricky or over-concentrate volatility in one account.) It's a balancing act but manageable. For TLH, avoid wash sales: remember it applies across all your accounts, including spouse's accounts if filing jointly – e.g., don't buy a stock in your IRA that you just sold for a loss in your taxable. Many accidentally trigger wash sales by reinvesting dividends – if harvesting around year-end, turn off automatic reinvest in that security. Also, TLH is not beneficial if you immediately plan to use the loss to offset a gain that would be taxed at 0% (wasted effort), but that's not common for high earners. For 1031 exchanges: the rules are strict. Identify replacement properties by day 45, complete by day 180, use a qualified intermediary – messing up any step can disqualify the exchange, making the sale fully taxable ⁴⁸. Also note: a 1031 doesn't eliminate tax, just defers – eventually if you cash out, you owe, unless you plan to hold until death (when heirs get step-up). Opportunity Zones: risk of the investments (they are often in developing areas, could be high risk), plus the deferral of the original gain only goes until end of 2026 (tax due in 2027) – so it's a short deferral now, and the main benefit is on the new gains which requires a decade hold. It's a long-term illiquid play. For CRTs: irrevocable and complex – you give up control of the asset, and the trust must be properly administered each year (tax filings, paying you income, etc.). It's a commitment to charity, so be sure that aligns with your values (the tax benefits are great, but you are giving away the principal eventually). Another big

consideration: **Net Investment Income Tax (NIIT)** 3.8% – planning strategies that lower your AGI can help avoid or reduce NIIT on investment income (like shifting income to tax-exempt bonds, or using losses to keep net investment income down). It's hard for a high-income surgeon to escape NIIT entirely (since it kicks in at \$250K AGI for couples), but if you retire and have a year or two with lower AGI, you might plan a sale in a year you drop below that threshold to avoid NIIT on that sale. On the flip side, be aware that NIIT also can hit passive income like rental income if you're above the threshold – unless you qualify as real estate professional (then rentals are non-passive and not subject to NIIT, but then they're active business income which if you materially participate, they might be subject to SE tax – but rental income generally isn't SE taxed if it's just rent). It gets complicated, but the point is to navigate these definitions with a tax advisor to ensure you're minimizing the 3.8% surtax where possible.

In sum, **Investment Tax Optimization** is about being as **efficient as possible with the dollars you've already earned and invested**. It doesn't require earning more or taking more risk – it's purely about keeping more of your investment returns in your pocket by cutting out needless tax leakage. Over a lifetime, this can add substantially to your wealth and accelerate reaching financial goals, all with relatively simple planning steps and awareness. It's the equivalent of improving the "margins" in a surgical practice – small percentage improvements that significantly boost the bottom line when compounded.

(Visual suggestion: a before-and-after bar chart could show an example: "Portfolio Value after 20 years – without vs. with tax optimization", illustrating maybe a 10-15% larger ending value with strategies applied. Another idea: a flow chart of tax-loss harvesting, showing how losses flow to the tax return (offset gains, then \$3K against income, rest carry forward). A map of the U.S. highlighting Opportunity Zones might be too detailed; perhaps a diagram of asset location: e.g., a piggy bank icon (taxable) filled with stocks, and a shield icon (IRA) filled with bonds and REITs, etc., to reinforce where to hold assets.)

Strategy 4: Practice Exit & Pre-Liquidity Planning

Objective: Minimize the tax impact when you convert your practice (or other business assets) into cash – i.e., when you sell your ownership, merge with a larger group, or otherwise "liquidate" the equity you've built. For many orthopedic surgeons who are partners or owners in their practice, the practice itself is a significant asset. Approaching retirement, you might be selling your share to younger partners, a hospital system, or a private equity roll-up. Similarly, you may be selling other related assets like your medical office building or equipment. **Pre-liquidity planning** means strategizing *before* the sale to structure it in the most tax-efficient way. This can involve the form of the sale (asset vs. stock sale), timing (spreading payments), and use of specialized vehicles (like we mentioned CRTs, etc.). It's a chance to potentially save hundreds of thousands in taxes on a one-time event.

Key Tactics for Practice Exit Planning:

- **Capital Gains vs. Ordinary Income – Structure of Sale:** The first principle is to strive for **capital gains treatment** on as much of the sale proceeds as possible, because long-term capital gains are taxed at 20% (plus NIIT) versus up to 37% for ordinary income ⁹ ¹⁰ . How do you do that? In a practice sale, this often comes down to **allocation of the purchase price** and the nature of what's being sold. For instance, if you sell stock of your practice entity (if it's a corporation), that sale generally yields capital gain to you (good). Buyers, however, often prefer an asset sale (so they can pick up depreciation benefits, etc.). In an asset sale, the price gets allocated to various assets: tangible assets (equipment, etc.), and intangible assets like **goodwill**. Tangible assets might trigger

depreciation recapture (taxed at ordinary or 25% rate for 1245/1250 property) – not great. But a large chunk often goes to goodwill, which for a personal service business is the *goodwill of the practice*. If your practice is a pass-through (S-Corp/partnership), goodwill sale is still capital gain to you ¹². If it's a C-Corp, sale of corporate goodwill is taxed at corporate level *and* again to you – which is awful double tax. This is where **personal goodwill** can come in for C-corp owners: you carve out the goodwill as **personally owned** by you, the surgeon (based on your personal reputation and relationships), and sell that portion individually, separate from the corporation ¹³. The IRS has allowed this in cases like *Martin Ice Cream* and others, provided no prior employment contract or non-compete had already transferred that goodwill to the company ⁵² ⁵³. In effect, the buyer pays you for personal goodwill (capital gain to you once) and pays the corp for any tangible assets (which can be handled). This avoids double tax on that goodwill portion, resulting in one layer of capital gains tax (maybe ~20–23.8%) instead of corporate 21% + individual 23.8%. For a large practice sale, that could easily save **20+ cents on the dollar** for that portion – e.g., for \$1M of goodwill, maybe ~\$200K saved ⁵⁴. It requires careful documentation (no existing non-compete, a specific purchase agreement for the personal goodwill) ⁵⁵ ⁵³, but it's a well-known strategy in professional practices. Even if not C-Corp, personal goodwill concept underscores negotiating how the sale is allocated. **Action item:** In any sale, work with your CPA and attorney when drafting the LOI and purchase agreement to influence allocation in a tax-favorable way. As seller, you want more toward **goodwill, personal goodwill, or stock**, and possibly less toward items like a consulting agreement or non-compete payment. Why? Because if the buyer insists on paying you, say, \$200K for a 3-year consulting or non-compete agreement, that \$200K is taxed to you as **ordinary income** (and subject to self-employment tax possibly), not capital gain. Sometimes a consulting period is part of the deal – you might negotiate to minimize that portion or fold value into the goodwill price. A non-compete payment is also ordinary income. It may be unavoidable to have those, but be mindful of amounts.

- **Installment Sales:** If you're selling to partners or a smaller entity that can't pay full price upfront, you might receive payments over several years (with interest) – this is an **installment sale** for tax purposes. Installment reporting means you recognize gain proportionally as payments come in, which spreads the tax hit. This can keep you from being pushed into a higher bracket in a single year. For example, selling for \$1M cash all at once might spike your income that year (especially if part is ordinary). But if you take \$200K/year for 5 years, you report 1/5 of the gain each year. This could keep you in a lower capital gains bracket (if you were near the edge of 15% vs 20%, though that's unlikely at these amounts), or at least avoid phase-outs or state bracket jumps. It also defers tax, which has a time value benefit (you pay as you get paid). Be careful: if tax rates are expected to rise, deferral could mean paying at a higher rate later. But currently, capital gain rates are stable, though ordinary rates might rise post-2025 if laws sunset – something to consider if a large portion would be ordinary (perhaps better to take that sooner under current lower rates). Also note, installment sales **do not help** for depreciation recapture or certain types of gains – those often must be recognized immediately in year of sale, even if payments are later. Also, if the buyer can pay upfront, consider whether *you* want to finance or not – taking installments means credit risk (will they pay you each year? ensure security/collateral). But from pure tax, it's a lever to spread income.
- **Timing the Sale (Calendar Year Planning):** If you have flexibility on when a deal closes, look at your other income for the year. For instance, say you plan to retire on June 30, 2026 and sell your practice then. In Jan–June, you'll have earned, say, \$400K of income. If the sale closes in July 2026, that sale adds to a year where you already had high income for 6 months. Perhaps you negotiate to close in early January 2027 instead. In 2027, you'd have zero or minimal work income, so the sale gains stand

alone – possibly avoiding pushing wages into higher brackets. (Keep an eye on the tax law: e.g., if the federal top rate goes up or the capital gains rate goes up in 2027 due to legislation, that could counteract the benefit – but assume stable rates, it might help to shift into a year with otherwise low income). Similarly, if you have major deductions one year (maybe a huge charitable gift deduction or a net operating loss carryforward), aligning a sale in that year could soak up the income with those deductions.

- **Use of Trusts or Retirement Vehicles at Sale:** We touched on **Charitable Remainder Trusts (CRTs)** earlier ⁵⁰ – this is directly relevant here. If you donate (technically, irrevocably transfer) part of your business or practice interest to a CRT *before* the sale, when the sale happens the CRT, being a tax-exempt entity, **pays no capital gains tax on the portion it sells** ⁵⁶. The proceeds then stay in the trust, you get an income stream for life from it (taxable as it comes out in tiers), and you get a current charitable deduction based on the remainder value that will go to charity ⁵⁷ ⁵⁸. This can be a brilliant move if you are charitably inclined anyway and worried about the tax on a sale. Essentially, you trade off leaving some portion to charity in the future in exchange for **tax-free compounding and immediate tax relief**. For example, Dr. Surgeon transfers 20% of her practice shares to a CRT, and keeps 80%. Practice sells for \$5M. The 20% (= \$1M) inside CRT pays zero tax; the 80% she held personally, she pays tax on. She also gets maybe a ~\$100-200K charitable deduction (depending on her age/payout rate) up front, which saves her maybe \$37-74K in current tax. And she'll get income for life from that \$1M (maybe ~\$50-60K a year if 5-6% payout). On the downside, that 20% of principal won't go to her kids – it goes to a charity she chooses after she passes (or after some term of years). But she may find that acceptable to reduce the tax hit and benefit a cause. This is complex and must be set up **before any binding sale agreement** (you can't do it after you sign a contract to sell – that would violate the “assignment of income” doctrine). So it requires foresight. **Another tool:** If you own your practice via a C-Corp and it has appreciated greatly (rare for service businesses, but maybe in some cases or if you had stock basis near zero), certain corporations *might* qualify for **Section 1202 Qualified Small Business Stock (QSBS)** exclusion. However, importantly, *healthcare services are typically excluded from QSBS benefits* ⁵⁹ (SSTBs are excluded categories for QSBS). So likely not applicable for a physician practice – but if you had a side business like a device company or software that you started, QSBS could exclude up to \$10M or 10x basis of gain (if held 5+ years). That's more entrepreneurial than practice-related, though.

- **Pre-Sale Entity Changes:** If time is on your side (5+ years out), consider if your current entity type is ideal for sale. For example, if you're in a partnership and thinking of selling to a corporate consolidator, maybe converting to an S-Corp could simplify stock sale (though can also sell partnership interests). If you're a C-Corp (maybe due to older rules or state laws), strongly consider electing S-Corp at least 5 years before sale to avoid some built-in gains tax issues when selling assets. Each scenario is unique, but an advanced tax advisor can simulate sale outcomes under different structures. Also, ensure corporate documents don't have provisions that impede tax-efficient deals – e.g., some old partnership agreements might mandate paying out accounts receivable to selling partners as ordinary income, etc. These little provisions can create unexpected taxes. Renegotiating them in advance (with partners) might allow a cleaner sale where buyer simply buys entity or assets for one lump sum.

- **Selling Your Medical Office or Equipment:** Many surgeons also own their clinic building or surgery center real estate. Selling real estate has its own tax considerations. If the property has appreciated, you face capital gains and “unrecaptured Section 1250” gain (basically depreciation taken, taxed at

25%). One strategy is a **1031 exchange** (discussed in Strategy 3) – perhaps you sell the building and exchange into a different investment property. If you're retiring, maybe you don't want property management duties – but you could 1031 into a passive vehicle like a Delaware Statutory Trust (DST) that holds commercial real estate managed by someone else, preserving the deferral. Or if charitably inclined, use a CRT for the real estate too. If you simply want out and want liquidity, be prepared for the tax and try to time it with the practice sale year or a year you can offset (maybe you had big losses elsewhere, etc.). If you are going to carry a note when selling building to younger doctors (installment sale), ensure interest is at least the AFR (to avoid imputed interest issues).

- **Prepare for Due Diligence and Optimal Valuation:** While not a pure tax move, a pre-exit strategy is to get your financials in order (clean books, clear distinction of personal perks vs business, etc.). Buyers will pay more for a practice with well-documented profits (EBITDA). Also consider if any **tax credits** or benefits you can capture pre-sale so that you, not the buyer, benefit. For example, if you haven't taken advantage of the **Employee Retention Credit (ERC)** or other pandemic-related benefits and you qualify, do it before sale (some deals might not attribute those to you if they close after a certain date). Also, if you plan large capital purchases, consider whether to do it right before sale (to use bonus depreciation and reduce taxes, but note it could also reduce EBITDA which might lower sale price depending on how buyer values it). There's a balance: you want to minimize taxes but also present a healthy business. A buyer may add back one-time expenses or discretionary stuff, but maybe coordinate with your valuation expert.

Who Should Implement This: Any surgeon expecting a **significant liquidity event** from selling a practice, surgical center shares, or related business property in the next 1–5 years. The greater the value of the sale, the more important the planning (small sales under, say, \$100K are more straightforward, but larger ones mid-six to seven figures deserve planning to shave rates from ~37% to ~20%). Also, those selling to an entity where they have negotiating power on terms (with a hospital you may have less flexibility, but you can still negotiate some terms). If your practice is being acquired by private equity, definitely get tax advisors on board early – those deals can be complex (you might roll over some equity, etc., which has its own tax pitfalls like ensuring a **tax-free rollover** under Section 351 or similar if possible).

Who Might Skip Some of This: If the sale is small or mostly goodwill and you're already an S-corp or sole proprietor, and it's basically a straightforward "walkaway" sale, you might not have much room for creativity beyond maybe installment sale and timing. Also, if you're a minority partner and the terms are largely set by others, you might not individually alter the structure, but you could still individually do things like CRT or QOZ with your proceeds. Someone who is simply closing their practice (not selling) – in that case, you're liquidating assets maybe at book value or selling off equipment – less tax impact beyond ordinary income recapture on equipment (though you might auction stuff for less than basis, in which case maybe a loss). But that scenario is more about making sure you plan for closing costs and such, not as much fun tax strategy.

Potential Tax Savings: This can be huge. For example, consider a \$2,000,000 practice sale structured poorly: if it's an asset sale with a lot allocated to a non-compete and AR buyout, you might end up with, say, \$500K taxed at 37% (ordinary) and \$1.5M at 20%. Tax bill roughly = \$185K + \$300K = \$485K (plus state perhaps). If structured well as mostly goodwill/stock: maybe \$1.9M at 20% and \$100K consulting at 37%. Tax = \$380K + \$37K = \$417K. That's a \$68,000 federal tax difference (plus saving Medicare taxes on that reallocated portion if relevant). If it was a C-corp scenario, the difference could be bigger (double tax vs single). E.g., \$1M goodwill via corp would net maybe only ~\$600K after double tax, vs. \$800K after single tax

if personal goodwill is used – a \$200K improvement ⁵⁴ . Using a CRT for a portion can completely eliminate tax on that portion (at the cost of giving the remainder to charity). A CRT or Opportunity Zone or 1031 can, in theory, reduce immediate tax on a big sale to near \$0, deferring or eliminating most of it – which could be hundreds of thousands saved (or at least deferred long-term).

Pitfalls and Things to Watch:

- **Step Transaction / Economic Substance:** Moves like personal goodwill and CRTs must be done correctly and not appear as sham just to dodge tax. Ensure legitimate business purpose and documentation. Personal goodwill needs evidence (no prior agreements tying your goodwill to the company, perhaps a narrative of how patients are loyal to you individually, etc.) ⁶⁰ ⁶¹ . The transaction documents should explicitly allocate to personal goodwill and be consistent on buyer and seller's tax forms ⁵³ . If the IRS smells that it was just re-labeling without substance (e.g., you had a non-compete that says all goodwill belongs to company, then you still claim personal goodwill, that won't fly ⁵²). Similarly, a CRT must be funded before any binding agreement to sell; if done after the fact, IRS can disallow and say you owe the tax because you had a prearranged sale.
- **Buyer's concerns:** Some strategies may conflict with the buyer's interests. For instance, buyers often want an asset sale so they can depreciate assets anew (they *like* paying for goodwill because they amortize Section 197 intangibles over 15 years ⁶²). They might be indifferent to your capital gains vs ordinary, except that if they allocate to consulting, it's deductible immediately for them, whereas paying for goodwill gives only a 15-year deduction. So they might push for more consulting/non-compete because that's immediate expense to them. You'll have to negotiate – maybe concede some but adjust price accordingly. If they are a hospital or non-taxable, they don't care about amortization, so you might have more flexibility (they might even prefer to call it a donation? Unlikely, but some creative deals happen, albeit careful of private inurement issues if non-profit).
- **State taxes:** Consider state implications. Some states tax capital gains and ordinary at the same rate, so the federal distinction might not matter for state, but others do (or have no income tax). Also, if you're selling to an out-of-state buyer or moving after sale, allocate appropriately for state sourcing. E.g., sale of goodwill of a practice is often sourced to the state where the practice operates. If that's a high-tax state, that's that. But if you move to a no-tax state before selling stock (since intangible, maybe taxed by residence), that could avoid state tax (though some states try clawback if it was built there). Don't commit tax evasion by shifting paper residence for a day just to sell – but if you legitimately were planning a move, the timing could matter.
- **Employee/Staff considerations:** If you structure as a stock sale, employees' status might just carry on. If asset sale, you might need to terminate them and buyer rehire – not directly a tax issue for you except possibly triggering payout of deferred comp or PTO (which could create deductions or income in final year for you). Just coordinate the logistics and costs (like paying out accumulated vacation is deductible but also uses cash, etc.). Possibly, you might pay stay bonuses or severances which are deductible to you and taxable to recipients – not really a planning point except to budget.
- **Retirement plan termination:** When you exit, if you had a pension/cash balance plan, terminating it and distributing assets can have timing considerations. Ideally, do it in a year you're not layering it on other big income. Also, ensure you handle any plan terminations in compliance (so you don't

create excise taxes, etc.). If you have an ESOP or something (unlikely in small med practice), that's another layer.

- **Accounts Receivable (AR) and Work in Progress:** In many medical practice sales, AR is handled separately – often the selling doctors keep the right to their AR (because buyer doesn't want to pay for money you already earned). Those AR then get collected post-sale and you get that as income. If you're a cash-basis taxpayer, that AR wasn't taxed yet – so guess what, as you collect it you have ordinary income. Plan for that. Some might try to accelerate collections pre-sale, or structure maybe a consulting where AR collected goes to buyer but they pay you something for it (but that likely remains ordinary anyway). It is what it is – just don't forget to include it in your tax projections. Same for any outstanding debts or liabilities you pay off (deductible perhaps then, which is good).
- **Non-qualified deferred comp pitfalls:** If you have any deferred comp arrangements (maybe a supplemental retirement plan in the practice), a change in control might trigger payouts. Those payouts could bunch income for you. If you can, maybe schedule them over a couple of years or start withdrawing a year before sale. Follow Section 409A rules for any changes (you can't just change payout timing last minute without 12-month/5-year rules). This is niche but applicable if you had, say, a deferred bonus program.

Exiting a practice is often a once-in-a-career event – tax planning here is truly where you can *create or lose* a lot of value. With foresight and good advice, you can ensure the lion's share of the sale proceeds end up funding your retirement, not the IRS's coffers.

(Visual suggestion: a flowchart of a practice sale could be helpful, showing two paths: one labeled "Standard Sale" with double tax for C-corp goodwill or ordinary chunks, and one "Planned Sale" with steps like personal goodwill, CRT, installment, etc., resulting in lower tax outcome. Another idea: a stacked bar chart showing how a \$1M sale might be divided (tax vs. net) under different strategies. Also, possibly a timeline graphic showing "N years before sale: do X (S-corp election?), 1 year before: set up CRT, identify goodwill, at sale: allocate price, after sale: invest proceeds tax-efficiently.")

Strategy 5: Legacy & Estate Tax Awareness

Objective: Preserve the wealth you've built for your heirs and chosen beneficiaries by being proactive about **estate and legacy planning**, especially in light of current estate tax laws. With multi-seven-figure net worths, orthopedic surgeons must be aware of both federal and state estate taxes, and implement strategies to reduce or eliminate those taxes where possible. The goal is to pass on assets *efficiently*, ensuring your family (or charities) receive the maximum benefit rather than a big slice going to the government. Also, beyond pure tax, legacy planning includes making sure your assets go *where* and *how* you want – which means proper wills, trusts, and beneficiary designations.

Key Considerations and Tactics:

- **Know Your Estate Tax Exposure:** As of 2025, the federal estate tax exemption is **\$13.99 million per person** (almost \$14M) ⁶³, or effectively \$27.98M for a married couple with portability ⁶⁴. Recent legislation (sometimes referred to here as OBBB 2025) actually made the historically high exemption **permanent at \$15 million, indexed** ⁶⁵. This means many surgeons will *not* face federal estate tax,

which is 40% on the value above the exemption ⁶⁶. However, be aware that if laws change or if your wealth grows beyond these thresholds, estate tax could be a factor. (Prior law would have sunset the exemption back to ~\$6M in 2026 ⁶⁷, but with the new act that appears to be averted; still, political winds can shift.) The key question: is your net worth (including life insurance, retirement accounts, real estate, etc.) likely to exceed the exemption when you and spouse pass? If yes or even close, you need strategies to **reduce the taxable estate**.

- **State Estate Taxes:** Several states have their own estate or inheritance taxes with much lower exemptions. For example, **New York** has an estate tax exemption of about **\$7.16 million** in 2025 ⁶⁸, roughly half the federal. Worse, NY has a “tax cliff” – if your estate is just 5% over the exemption (~\$7.52M), you lose the exemption entirely and the whole estate is taxed up to 16% ⁶⁹. Other states: Illinois (~\$4M exemption) ⁷⁰, Massachusetts (\$1M exemption), Maryland, Oregon, etc. Many surgeons practice in states like NY, NJ, CA – note NJ repealed its estate tax (now \$0 exemption because no tax), but NJ still has an inheritance tax for distant heirs ⁷¹. The point is, **check your state’s rules**. It’s very possible to avoid federal estate tax but still owe, say, 10–16% to your state on amounts over a few million. Planning might involve *where* you retire. For instance, Florida has **no state estate tax** ⁷² (and no income tax). A number of physicians do relocate to FL or TX in retirement for climate and tax reasons. While you shouldn’t move solely for this, if it’s already on the table, the estate tax angle is a plus. Alternatively, if staying put in a state like NY, you must pay attention to that cliff – e.g., an \$8M estate in NY could incur roughly a \$1M state tax, which might have been mostly avoided by gifting or planning down to \$7M.

- **Basic Estate Planning Documents:** This isn’t about taxes per se, but a legacy plan starts with **a will, powers of attorney, healthcare directives, and likely a revocable living trust** (especially if you have property in multiple states or want to avoid probate complexities). Ensure beneficiary designations on retirement accounts and life insurance are up to date and align with your will/trust (these assets pass outside the will directly to named beneficiaries). Surprisingly, many high earners forget to update these – causing ex-spouses or unintended folks to receive assets, or assets to go outright to young heirs instead of in trust. From a tax perspective, a **properly drafted will/trust** for a married couple can also use both spouses’ federal exemptions fully if portability isn’t used, or in states without portability (e.g., New York does *not* allow porting the \$7M exemption to spouse ⁷³). That typically means using a “**credit shelter trust**” (bypass trust) at first death to shelter the first exemption. Given portability at federal level, some couples rely on that instead (by filing an estate tax return when first spouse dies to preserve unused exemption). However, trust planning can still protect growth from estate tax and also control distribution to heirs (useful if second spouse might remarry or you have children from prior marriage).

- **Lifetime Gifting:** One straightforward way to reduce your taxable estate is to **give assets away while you’re alive**, especially if you’re comfortably funded for retirement. You have an annual gift tax exclusion of **\$17,000** per donee in 2023 (indexed, \$19,000 in 2025 ⁷⁴). If you and spouse have 3 children, you could shift \$17K x2 x3 = \$102K per year to them without even denting your lifetime exemption. Over 10 years that’s over \$1M out of your estate (plus any appreciation on those gifts is also out – a key benefit: gifting assets that will grow substantially moves future growth out of your estate). You can also directly pay tuition or medical expenses for someone without it counting as a gift – a strategy if say you want to pay grandchildren’s private school or help a parent with medical bills, do it directly to the institution and it’s not a taxable gift. For larger gifts, remember you have that **\$13.99M lifetime exemption** to use (minus any used at death). If your estate is likely to exceed

the exemption, you might want to **use some of it now** by gifting to heirs or to irrevocable trusts. Especially given it's historically high – some worry it could be reduced by future legislation. If you're, say, worth \$30M, you might gift \$10M now to a trust for kids, using up part of your combined \$27.98M couple exemption, reducing your estate and any future taxable amount. The assets and their future growth escape estate tax (provided you survive 3 years if dealing with state like NY gift add-back, and 7 years for some inheritance taxes in other countries, etc.). Even if you're not that wealthy, consider strategic gifts: maybe gifting a vacation home now to kids (or to a trust) while its value is moderate, so that future appreciation happens out of your estate (and maybe if estate tax fell back to \$5M in distant future, you've removed that asset from count).

- **Trusts for Heirs:** Beyond the revocable living trust you might use to avoid probate, **irrevocable trusts** can be set up to hold assets for heirs in a way that keeps those assets out of your estate (if properly structured) and potentially out of *their* estates too. For example, a **Spousal Lifetime Access Trust (SLAT)** – you gift, say, \$5M to a trust; your spouse is a beneficiary during their lifetime (so you indirectly still have access if needed via spouse), and at spouse's death it goes to kids. The \$5M (plus growth) is outside your estates, but you haven't fully relinquished benefit because your spouse can use it if needed. There are complexities (each spouse can't set up identical SLAT for the other – or they risk the "reciprocal trust doctrine"). But it's a popular move especially when exemptions were slated to drop – many did SLATs to lock in use of the \$11M+ exemption while it was available. Under the new law making \$15M permanent, the urgency is less, but diversifying wealth into an irrevocable trust can still hedge against future law changes. Also, such trusts can be designed as **"dynasty trusts"** (lasting for generations, avoiding estate tax for your grandkids and beyond, within whatever a state law allows perpetually).
- **Life Insurance for Estate Liquidity:** A common strategy for higher estates is using life insurance to provide liquidity to pay estate taxes or to equalize inheritances. For it to be estate-tax free, it should be held in an **Irrevocable Life Insurance Trust (ILIT)** or owned by others. For instance, if you have a large illiquid asset (like a medical practice building you kept or a family compound) and you want to leave it to one child, you could have an insurance policy that pays cash to another child or to cover the tax so the asset doesn't have to be sold. Premiums you gift to the ILIT count against gift limits (though often structured under annual exclusion if done right). The death benefit, if ILIT owns it, won't be in your estate. This is more of a tool to manage liquidity and fairness, not directly reducing tax except by providing cash to pay it so assets don't have to be fire-sold.
- **Estate Tax Clawbacks and Changes:** Keep aware of legislative changes. The content suggests the estate exemption is fixed high by OBBB. But always plan with flexibility – use trusts that can adapt (distribute out if needed, etc.). If you have potential to exceed exemption down the road (e.g., you're at \$10M now, could be \$20M in a couple decades), consider strategies like **GRATs (Grantor Retained Annuity Trusts)** – a technique where you put assets likely to appreciate (say stock or even value of your practice pre-sale) into a trust that returns you a fixed annuity; any excess growth beyond a certain IRS rate passes to heirs tax-free. This is great for assets expecting big jump (like a practice about to be acquired by PE – one could theoretically GRAT some interest *before* the deal so that the value uptick goes to heirs outside estate). The tax law currently allows very efficient zeroed-out GRATs, but this has been on the chopping block occasionally.
- **Generational Planning & Asset Titling:** If you have significant wealth, also consider **generation-skipping** – e.g., leaving assets in trust for not just kids but grandkids, leveraging the **GST tax**

exemption (which is equal to the estate tax exemption, ~\$14M). This avoids another layer of estate tax when your children die. Many comprehensive plans set up trusts for kids that last their lifetimes and then go to grandkids, using GST exemption to protect it. This way, the wealth you leave can benefit multiple generations without being taxed at each generational level.

- **Charitable Legacy:** If you have philanthropic goals, incorporating them in estate plan can reduce estate tax and perpetuate your legacy. Charitable bequests in your will are estate-tax deductible (so effectively tax-free transfers to charity). If you know you want to give a portion to charity, doing so at death can zero out tax on that portion. Some people even aim to leave whatever would be taxed to charity rather than to IRS. Example: if your estate is \$5M over the exemption (which would incur ~\$2M tax), you could instead give that \$5M to a foundation/charity – then no estate tax and the charity gets it rather than IRS. Depends on your wishes. Tools like **Donor-Advised Funds or Private Foundations** can be established either during life or by your will to direct charitable funds, and also serve as vehicles for your family to continue philanthropic work in your name.

Who Should Focus on This: All high-net-worth surgeons should at least cover the basics: up-to-date estate documents and beneficiary designations. Those with projected estates above (or even near) the exemption should engage in estate tax mitigation strategies (like gifting, ILITs, SLATs, etc.) ideally *sooner* rather than later. If you're in a state with an estate tax and your estate exceeds that lower threshold, you'll need planning specifically to avoid state taxes (like making gifts to reduce estate below the cliff, or setting up trusts that can do some state tax planning). For example, New York's estate tax cliff might motivate someone with a \$8M estate to gift \$1.5M out to bring it to ~\$6.5-7M – saving potentially hundreds of thousands in NY tax ⁶⁹. Typically, estate planning becomes more pressing as retirement nears or when major life events happen (marriage, divorce, grandkids, etc.), but from a tax perspective, planning earlier can save more (because of asset appreciation out of estate, etc.).

Who Might Not Need Complex Planning: If your estate including insurance is well under the federal and state exemption and likely to stay so, you won't owe estate tax. In that case, focus might be on non-tax issues: ensuring the right people inherit and possibly asset protection or guardianship for minor kids, etc. But not every surgeon is ultra-wealthy – some might have, say, \$3M net worth and no state estate tax – they won't owe estate tax. They should still avoid probate tangles and make things easier for family, but they don't need GRATs or ILITs for tax reasons (though asset protection trusts could be a separate reason, e.g. if worried about future lawsuits beyond malpractice coverage, etc.).

Benefits of Planning: The difference can be stark. Without planning, an estate that exceeds the exemption can lose 40% of the excess to federal tax. With planning (like using both spouses' exemptions fully, making lifetime gifts), you can potentially have **\$28M+ per couple pass estate-tax free** ⁷⁵ ⁷⁶. For state taxes, planning around a cliff can mean paying 0% vs. 16% on the whole thing. Example: A NY surgeon has \$8M estate, no planning -> estate ~\$8M triggers ~16% on all \$8M (because of cliff), tax roughly \$1.2M. With planning to be at \$7M, \$0 tax. That's \$1.2M saved for heirs ⁶⁹. Gifting strategies can also shift future growth out, which might save even more in the long run. ILITs ensure, say, a \$5M life policy doesn't bloat your estate by that at death (saving potentially \$2M tax if it would've been taxable). A well-structured plan can virtually eliminate estate tax for estates at or somewhat above the exemption, and significantly reduce it for much larger estates (though beyond certain high levels, some tax might be inevitable without major charitable giving).

Pitfalls and Risks:

- **Over-gifting:** Don't give away assets you may later need – longevity and medical costs can be uncertain. Once you put assets in irrevocable trusts or give outright, they're gone (except trusts where spouse is beneficiary, but even then, if spouse dies before you, that access is gone). Balance tax planning with personal financial security. It's often said "don't let the tax tail wag the dog" – for estate tax, that means ensure you keep enough for comfortable lifestyle before aggressively gifting just to save estate tax.
- **Changing laws:** Relying on a high exemption being permanent could backfire if laws change with future administrations. Ideally, plans should be flexible – e.g., give trustees discretion, use formula clauses that adjust if laws change (like if exemption is lower, how to fund trusts might change). Some older documents have outdated formulas referencing credit amounts that no longer make sense with today's high exemptions, which could unintentionally disinherit a spouse or overfund a trust. *Action:* Review old wills/trusts from years ago to ensure they still do what you intend under current law.
- **Trust administration:** Trusts add complexity. Trustees must manage assets and follow terms. Also, certain trust strategies (GRATs, QPRTs) have mortality risk – if you die during the term, the intended benefit may be lost. For example, if you set a 10-year GRAT and die in year 8, the assets are basically back in your estate. So consider health and age in choosing strategies (maybe shorter term GRATs repeatedly for older grantors, etc.).
- **State considerations:** If you move states, your estate plan might need updates (different state laws on trusts, community property vs. common law states affecting spousal share, etc.). Always coordinate with an estate attorney in your state.
- **Life insurance:** If using ILITs, make sure to actually transfer the policy to the trust and have someone (trustee) pay premiums using gifts you give – many ILITs fail because they weren't administered right (e.g., you kept paying premiums personally – which can pull the policy back into your estate as an incident of ownership potentially). Also, if you die within 3 years of transferring a policy out of your name (to an ILIT), it's pulled back into estate by IRS rules. To avoid that, sometimes the ILIT itself should purchase a new policy rather than you transferring an existing one – or at least be aware of the 3-year rule.
- **Communication with heirs:** As part of legacy, consider how your heirs will handle the wealth. Trusts can protect them from their own mismanagement or divorces/creditors. But also, leaving some instructions or guidance (perhaps in a letter of wishes) can be valuable. From a tax side, educating them on inherited IRAs (which now must be emptied in 10 years for non-spouse – no more lifetime stretch, so plan for that), or basis step-up (tell them which assets have huge built-in gains so they know not to sell right before you die!).

One quick note on **step-up in basis:** At death, most assets get stepped-up basis to date-of-death value (meaning heirs can sell immediately with little capital gains tax). This is a powerful benefit; it often outweighs trying to give assets to kids during life for cap gains reasons. For example, you have stock worth \$2M, basis \$200K. If you give it to child now, you use part of exemption (no estate tax on that gift presumably), but child also takes your \$200K basis – if they sell, big capital gain tax. If instead you hold it

until death (assuming estate tax not an issue), the child inherits at \$2M basis – they could sell immediately with zero cap gain. So sometimes *not* gifting highly appreciated assets is smart (let the step-up happen). But if your estate is taxable, sometimes you intentionally give away low-basis assets to get future appreciation out – it's a trade-off: avoid estate tax (40%) vs. child potentially pays cap gain (20-23%). Usually estate tax is bigger threat if applicable. But if estate tax likely nil, better to hold and let them get step-up. So coordinate estate planning with income tax planning.

In closing, **Legacy & Estate Tax Awareness** is about ensuring the fruits of your lifetime of work go where you want in the most efficient way. It requires periodic review as laws and family situations change. It's not a set-it-and-forget-it, but the time invested in planning can yield peace of mind and potentially save your family a fortune in taxes and headaches later.

(Visual suggestion: a pyramid chart showing the estate planning process: Base = Will/Trust/POA (foundation), next level = Life insurance/beneficiary forms, next = gifting strategies, top = advanced trusts/GRATs etc for ultra-high net worth. Alternatively, a map highlighting states with estate tax and their exemptions to alert the reader if they're in one. Another useful visual: a timeline depicting how an inheritance flows with vs. without planning – e.g., "Dr. X dies with \$20M estate, no plan: \$X to IRS, \$Y to family vs. With planning: \$0 IRS (or minimized) \$Z to family/trusts, \$ to charity.")

Case Studies: Tax Strategies in Action for Practice-Owning Surgeons

Let's illustrate how these strategies can come together with a few hypothetical (but realistic) case studies. Each case study profiles an orthopedic surgeon (or couple), outlines their situation and challenges, then shows the strategy applied and the tangible impact.

Case Study 1: Maximizing Retirement Contributions and Cutting Taxes Late-Career

Profile: Dr. Alice, age 60, is an orthopedic surgeon in private practice (S-Corp) in California. Income \$1.1M/year. Married to a non-working spouse, two adult children. Practice has 5 employees. Alice has about \$4M in her 401(k)/IRA, but she's behind on her retirement goal and wants to retire at 65. Net worth ~\$8M (including home). High state taxes (CA ~13%). No estate tax concern federally (under exemption), but California has none anyway.

Core Issue: Alice's current saving via a 401(k) profit-sharing plan is **capped at ~\$67,500/year**, which hardly dents her taxable income. She's paying large taxes – ~37% federal + 3.8% NIIT + 13% CA = over 50% marginal on top dollars. She's worried she hasn't saved enough given her spend rate and looming retirement. Also, her practice income keeps her in NIIT territory and drives high state taxes.

Strategy Applied: In consultation with a financial planner, Alice implements a **Cash Balance Pension Plan** for her practice (Strategy 2). At 60, she can contribute approximately **\$280,000 per year** into the cash balance plan (while still contributing \$67.5K to the 401k) ²⁷ ⁷⁷ . Total pre-tax contributions ~\$347,500. To make this work, she uses a "graded" contribution formula that gives older owner (herself) a much larger allocation relative to employees – the plan is designed such that employees get a 5% of pay contribution (which, for 5 staff averaging \$60K, is \$15K total), and she (the owner) gets the lion's share. Combined with existing profit-sharing, employees already got 5% safe harbor + 2% profit-sharing, so an additional small percent into cash balance satisfies nondiscrimination tests ³² ⁷⁸ . All in, employees receive an extra \$20K/

year in contributions (which is a cost to the practice, but deductible). Alice redirects a big chunk of her income into the plan.

She also reviews her **S-Corp salary vs. distribution** and realizes she has room to lower her W-2 salary from \$500K to \$300K given her practice's profit (still reasonable for an ortho surgeon's salary). This increases her K-1 distribution by \$200K, on which she saves the 3.8% Medicare + Additional Medicare tax (about **\$7,600 saved**). ²¹ ²²

Additionally, she and her spouse execute a **backdoor Roth IRA** each year (\$7,500 each at age 60, \$15K total) – it's small, but over 5 years that's another ~\$75K in Roth funds grown for later.

Tax Impact: The huge retirement contributions cut Alice's taxable income dramatically. Instead of \$1.1M, she now has roughly \$752K taxable (because ~\$348K went into tax-deferred retirement). That likely drops her from the 37% bracket to maybe the 35% bracket for federal. Federal tax saved ~37% of \$348K ≈ **\$128,760** that first year (and similarly each year, adjusted if income changes). CA tax ~13.3% on that amount would be another ~\$46K saved. So about **\$175K/year tax savings** by leveraging the cash balance plan. Over 5 years, that's ~\$875K direct tax saved (not even counting investment growth). The \$7.6K Medicare tax saved via S-Corp adjustment each year adds another ~\$38K over 5 years. So in five years to retirement, Alice might save around **\$900,000 in taxes**.

Meanwhile, she'll have stuffed roughly \$1.7 million of pretax money into her retirement accounts over those 5 years (plus growth). Her retirement nest egg gets a significant boost – bringing her 401k/pension balance closer to ~\$6M by retirement (assuming moderate growth). At 65, she rolls the cash balance and 401k into an IRA. She plans to convert some IRA to Roth in the few low-income years after retirement (before RMDs and while in a no-income-tax state, as she plans to relocate to Nevada).

Audit/Mistake Avoided: She was careful to pay herself a **reasonable salary** of \$300K ¹⁸ (given median ortho salaries ~ \$600K, \$300K as W2 is defensible since she also gets K-1 profit – total still ~\$1.1M). She hired an actuary to administer the cash balance plan and committed to funding it for at least 5 years ³¹ (so IRS sees it as bona fide, not just a 1-year dodge). Her plan passed IRS nondiscrimination by giving employees slightly enhanced 401k matches and the modest cash balance accrual (they ended up with ~10% of contributions, she got 90% – within legal limits ⁷⁸).

Result: Alice reaches retirement at 65 far more financially secure. Her tax savings effectively helped fund an extra ~\$1M into her accounts (in taxes not paid and invested instead). She reduced her effective tax rate in those years and managed to accumulate enough to maintain her lifestyle. As a bonus, by moving to Nevada on retirement, she avoids CA tax on withdrawals and can convert some IRA to Roth at perhaps a 24% bracket instead of 35% (another future tax arbitrage). This case shows how **maxing out advanced retirement contributions** and entity tweaks can transform a late-career financial picture.

Case Study 2: Tax-Savvy Practice Sale and Retirement Transition

Profile: Dr. Brian, age 63, is a founding partner in OrthoGroup, a 4-surgeon practice (structured as a **C-Corp** professional corporation). Each partner owns 25%. A hospital is acquiring the practice to form an employed group. Purchase price allocated to the practice (goodwill, assets) for Brian's share is \$2 million. He also owns personally the surgery center building (50% share) which he plans to sell in a separate deal for \$1 million

(his share). Brian's wife is 62, they plan to retire and move to Florida post-sale. Net worth before sale ~\$12M (excluding practice), including \$3M IRA, \$4M in brokerage, \$2M real estate (home + rental), etc.

Core Issue: If Brian sells his C-Corp shares via asset sale, the practice sale proceeds will be taxed at corporate level **and** again as dividend – potentially ~double taxation totaling ~50+% ⁵⁴. On \$2M that could be ~\$1M tax. Plus the building sale \$1M will incur capital gains (depreciation recapture and 20% cap gain). He's also on the cusp of federal estate tax (post-sale net worth might hit \$15M+ which is near exemption). He wants to maximize what he keeps, provide for his children, and also has charitable intent (he's long wanted to endow a scholarship at his alma mater).

Strategy Applied: Pre-Liquidity Planning in full force:

- First, Brian's attorney identifies that much of the practice's value is tied to **Brian's personal reputation and referral relationships** – classic *personal goodwill*. There were no non-competes in place between Brian and his C-corp (they never had him sign one). So they structure the deal such that out of the \$8M total practice value, a substantial portion (say 50%) is allocated to personal goodwill of the four doctors. Brian, thus, will sign a personal goodwill sale agreement with the hospital for \$1M (his 25% of the allocated goodwill) ¹³. The hospital pays that directly to him in exchange for his personal goodwill (and a new non-compete as part of the deal, to transfer that goodwill). The remaining corporate assets (\$1M to Brian's share) go through the C-corp. Result: Brian gets \$1M as long-term capital gain directly (only ~20% tax), and the corp gets \$1M for other assets (which will be taxed at 21% corp, then the remainder as dividend to him at 20%). Roughly, personal goodwill saves him from double tax on that half of the sale ⁵⁴ – perhaps ~\$200K+ tax savings net.
- Second, Brian anticipates the building sale will net ~\$1M to him with about \$400K gain. Instead of selling outright and paying maybe ~\$95K tax (federal + state on recapture and gains), he decides to **1031 exchange** his 50% interest into an income property in Florida (through a DST). This defers the tax entirely ⁴⁶. Now he has a replacement property generating income for retirement, and if he holds it until death, his heirs will get a step-up and never pay the deferred capital gains tax. Alternatively, he could have used a Charitable Remainder Trust for the building, but he preferred to preserve it for family and the 1031 achieves that with deferral.
- Third, Brian and his wife take advantage of **Florida residency** once they move: no state income tax on the practice sale or retirement withdrawals. (They establish domicile there before year-end of sale, which in this case saves them, say, 6-10% state tax they'd owe if they stayed in their original state – if from NY, could be near 10% on these gains. So moving saves easily **\$100K+** in state taxes on combined transactions.)
- Fourth, they set up a **Charitable Remainder Unitrust (CRUT)** and fund it with \$500,000 of highly appreciated stock from his brokerage *before* selling those investments to raise cash for retirement. This avoids immediate capital gains on, say, \$300K of embedded gain ⁷⁹. The CRUT will pay them 5% annually (~\$25K) for life, and then go to charity (his scholarship fund) at death. He gets an income tax deduction of about ~\$150K now for the charitable portion ⁸⁰, which he uses to offset some of the practice sale ordinary income (like the dividend portion) that year. Essentially, he's pre-paying some charity to reduce his current tax and securing a legacy gift.

- Fifth, concerned about estate size, Brian and wife establish a **Spousal Lifetime Access Trust (SLAT)**. Right after the sales, they gift \$6M into a trust for the benefit of wife and children. This uses up \$6M of their combined estate exemption (which is okay, plenty left). That \$6M (plus growth) is now outside their estate. If their remaining assets grow, they are more comfortable that by the time they pass, their estate might be around \$10M which is below likely exemptions (especially with \$15M exemption now). The SLAT is invested and may even purchase a life insurance policy to leverage the gift (an ILIT could be combined, but here maybe not needed).

Tax Impact:

- **Practice Sale:** By allocating \$1M to personal goodwill, that portion is taxed at 20% = \$200K. The other \$1M through C-corp: Corp pays 21% (\$210K), leaves \$790K to distribute as dividend taxed 20% (\$158K). Total tax on that portion ~\$368K. Combined practice sale tax ~\$568K. If he had not done personal goodwill and all \$2M went through C-corp, tax would've been roughly $\$2M * (21\% + (79\% \cdot 20\%)) \approx \$2M(21\% + 15.8\%) = \$2M \cdot 36.8\% = \$736K$. So he saved about \$168K* in taxes with the personal goodwill strategy ⁵⁴.
- **Building Sale:** He deferred the entire ~\$95K tax via 1031. That's money in his pocket (or rather, not paid out) that he can invest. In present value, that's a \$95K savings (and if held until death, a permanent savings effectively).
- **Charitable Trust & Deduction:** The \$150K immediate deduction from funding the CRUT saves him about $\$150K * 37\% \approx \$55K$ in federal tax (plus no state tax since in FL). It also eliminated capital gains on the \$300K embedded gain – saving a further ~\$71K in cap gains/NIIT. Those assets now grow tax-free in the trust to pay him income. In return, perhaps ~\$500K (plus growth) will go to charity at their deaths, which he's happy to do. And his scholarship will be funded substantially. In essence, he traded a part of his estate (which would have been taxed 40% if left in estate or 20% if sold) to benefit charity and got tax perks now and income for life.
- **Estate Reduction:** By gifting \$6M to the SLAT, they removed that plus all future appreciation on it from their estate. If that \$6M grows to \$12M by the time both die, that whole \$12M escapes estate tax. If they had kept it, at 40% estate tax that would be \$4.8M tax. True, current law gives them \$15M each exemption, so maybe they wouldn't have been taxed – but given inflation of assets or law uncertainty, they wanted to be safe. Now their remaining estate is ~ \$2M (house) + \$4M IRA + \$1M 1031 property + \$2.5M other investments = ~\$9.5M, which is under one exemption. If exemptions stay high, no estate tax; if they drop, they have cushion. Essentially, they **future-proofed** against estate tax and also set up a legacy trust for kids. (The SLAT could also buy life insurance in an ILIT structure – but they might not need to if estate tax not expected.)

Overall Outcome: Brian effectively *kept* a much larger share of the sale proceeds and positioned his wealth in a protective, legacy-friendly way:

- Without planning: he might have netted maybe \$1.3M from practice after double tax, \$0.9M from building after tax, and left everything in his estate taxed later.
- With planning: he nets about \$1.43M from practice (out of \$2M, thanks to less tax), a full \$1M reinvested from building (no tax hit), got \$55K current tax savings from CRUT + avoided \$71K cap gain, and he is set to pay zero state tax by moving. That's roughly **\$300K+ immediate tax reduction**

across these moves (168K + 95K + 55K + 71K, though the CRUT gain avoidance overlaps with estate/charity trade-off). More importantly, his estate down the line will save potentially millions by the trust planning and the fact that assets that did get taxed (practice sale) he's spending or have removed.

- The family is financially secure with diversified assets (rentals, trusts, etc.), and his philanthropic goals are met in a tax-efficient manner.

Mistakes Avoided: He nearly went with a straight asset sale via C-corp – which would have been very costly. Thankfully, advisors informed him of personal goodwill case law (e.g., *Martin Ice Cream*, etc.) so he could justify that allocation ⁸¹ ⁸². They also made sure to draft and sign the personal goodwill purchase agreement **before** the corporate asset sale closes, with consistent allocations ⁵³. For the CRT, they ensured it was established and funded with stock **prior** to selling those stocks (and certainly before death!). For the 1031, he followed the 45-day/180-day rules strictly with a qualified intermediary ⁸³. The SLAT was set up while both spouses are alive and healthy to maximize use. They are mindful to avoid “reciprocal trust” issue – only Brian made the SLAT for wife/kids; wife did not make a similar trust for Brian (which would negate the benefit). Also, they are keeping records to prove Florida residency (to avoid any challenge from their old state on income tax – e.g., changed voter reg, driver's license, primary home, etc.).

This case demonstrates a full-spectrum approach: **practice exit planning, investment deferral, charitable giving, and estate planning** combined. It shows how a complex situation can be navigated so that Dr. Brian retires with his wealth intact (and in fact enhanced for heirs and causes), rather than seeing a large chunk disappear in various taxes.

Case Study 3: Mid-Career Course Correction – From Overpaying to Optimized

Profile: Dr. **Carol**, age 50, is a busy orthopedic surgeon in a group practice (LLC taxed as partnership) in New Jersey. She earns about \$800K. She hasn't done much tax planning – she maxes a SEP-IRA (\$58K/year) and typically just takes the substantial tax hit each April. Net worth \$3M, on track but not extraordinary. Practice has 10 employees (the group handles their 401k etc., she just participates). She also has some side income (\$100K/year) from consulting for a device company (1099 income) which she's been reporting on Schedule C.

Core Issues: Carol is **paying too much in taxes unnecessarily**: - Her consulting 1099 income (\$100K) is all subject to self-employment tax (roughly \$15.3K) because she's a sole proprietor for that. - She lives/work in NJ, a high-tax state (state rate ~10.75% for her bracket) but isn't utilizing the NJ **Pass-Through Business Alternative Income Tax (PTET)** workaround for SALT. - She has substantial taxable investments (\$1M) but never considered asset location or harvesting – last year she paid \$50K in capital gains tax after selling some funds to buy a shore house. - She also had been using only a SEP-IRA for retirement, not realizing she could possibly do a **Cash Balance Plan** or use a **401k in her side business** to stash more.

She doesn't plan to retire for 15 years, so she has time to adjust course. She feels she's “leaking” money via taxes that could instead go toward college for her kids and future savings.

Strategy Applied: Carol undertakes a multi-pronged “tax tune-up”:

- She **forms an S-Corp for her consulting business** (“Carol Ortho Consulting Inc.”). She routes her device consulting income through it. She pays herself a reasonable salary of \$40K and takes \$60K as distribution. This immediately saves her ~3.8% on \$60K (~\$2,280 Medicare tax saved yearly), and also

likely reduces audit risk by not filing a large Schedule C ⁸⁴ ⁸⁵ . The S-Corp also lets her adopt a **Solo 401(k)** for that income stream: she defers \$19.5K as employee and the S-Corp contributes ~25% of her wages (\$10K) as employer – about \$29.5K more sheltered pre-tax (SEP-IRA would have allowed at most \$20K on \$100K profit, so she squeezes ~9.5K more plus she can do catch-ups when 50+). Over 15 years that extra saving compounding matters.

- Within her practice LLC, she advocates to elect the NJ **Pass-Through Business Alternative Tax**. The practice opts in, paying NJ tax at entity level so it can deduct it federally. For her, that means her share of NJ taxes (say \$60K on her income) becomes a federal deduction to the partnership ²⁴ instead of being lost to the SALT cap. At 37% fed bracket, that's about **\$22K federal tax saved per year** via restored deduction. (NJ gives her a credit on personal return for the entity tax paid, so she isn't double-taxed.)
- She adjusts her investment placement. She moves her high-yield bond fund from taxable into her IRA (swapping with an equivalent amount of stock index fund from IRA to taxable). This could save her maybe ~2% of that bond interest in taxes annually (say \$500 interest, taxed 37% vs deferred 0, small but each year adds up). She also starts **harvesting losses** in her taxable account opportunistically. In the 2020 market dip, for example, she harvested \$100K of losses, which she used to offset rebalancing gains and took \$3K against income, saving maybe \$24K in taxes that year (federal+state). She sets a rule to review harvesting each quarter.
- She realized she could actually contribute more to retirement via her practice too: the group's 401(k) allowed after-tax contributions. She begins doing the **Mega Backdoor Roth** in the group plan – contributing an additional \$20K after-tax and converting it in-plan to Roth. This doesn't save current tax (after-tax contributions), but it builds Roth assets for later which will grow tax-free.
- She also uses **529 plans** for her kids' college savings, getting a small state tax deduction and growing funds tax-free for education (not big dollars, but every bit counts and it segregates that goal).

Impact:

- **Self-Employment Tax Savings:** S-Corp consulting move saves ~\$2.3K/year as noted. Over 15 years, that's ~\$34K saved, plus additional ~\$9.5K *pretax* saved into solo 401k yearly. So that's an extra ~\$142K contributed over 15 years, which might grow to ~\$200K by retirement.
- **SALT Workaround:** \$22K/year federal tax saved from PTET deduction. Over 15 years (assuming stable rules) that's ~\$330K saved.
- **Investment Efficiency:** By harvesting, she expects to save maybe \$10K per year on average in capital gains taxes (some years more, some less). If markets are volatile, could be more. But conservatively ~ \$150K saved over 15 years via capital gains management. And by shifting asset location, maybe another 0.3% improvement on \$1M portfolio = \$3K/year, ~\$45K over 15 years in after-tax growth benefit.
- **Retirement accumulation:** Not directly tax saved, but the Mega Backdoor Roth contributions of \$20K/year for, say, 10 years until kids college, yields \$200K put into Roth that grows tax-free –

providing a nice tax-free bucket in retirement, potentially worth \$300K+ by then. Also, her additional solo 401k contributions (above what SEP allowed) and catch-ups add tens of thousands to pretax savings.

- **Total Tax Reduction:** Summing rough 15-year savings: \$34K (SE tax) + \$330K (SALT fix) + \$150K (harvesting) + \$45K (asset location) = **\$559K** less in taxes paid. This is money that instead went to investments, college, or lifestyle.

Carol effectively gave herself a series of “raises” by keeping more of her money. She was at risk of burnout partially due to feeling that “half my extra effort goes to taxes.” Now she sees tangible rewards for efficiency: She funded her kids’ college largely from the tax savings, and bolstered her retirement accounts. Also, by the time she’s nearing retirement, her estate will likely be under any tax thresholds since she’s using assets for those goals (and NJ has no estate tax now, though they have an inheritance tax for non-lineal heirs – she’s leaving to kids so that’s exempt).

Pitfalls Avoided: Carol got professional help to ensure her **S-Corp salary** was not unrealistically low – \$40K on \$100K revenue is reasonable for consulting (since it’s not surgery income, and she still has to personally do the consulting – arguable either way, but should be okay) ⁸⁶ ⁸⁷. She also ensures to keep separate books for S-Corp and file payroll, etc., to avoid any compliance issues. For the SALT workaround, New Jersey’s rules are followed (elections filed timely). Her advisor monitors that the PTET doesn’t inadvertently raise her NJ tax beyond credit (it generally shouldn’t, it’s beneficial). With harvesting, she is mindful of **wash sale rules** – if she sells a fund at loss, she swaps to a slightly different fund (e.g., sells S&P500 ETF, buys Total Market ETF) to avoid wash sale ³⁶ ³⁸. For the Mega Roth in her group plan, she confirmed the plan allowed in-service rollovers and after-tax contributions; she also times the conversion quickly to avoid meaningful taxable earnings on those after-tax contributions.

Result: A mid-career surgeon who felt she was overpaying fixed many leaks: business structuring, state tax strategy, and investment tax management. Her effective federal tax rate dropped a few percentage points. She feels more in control: rather than dreading tax time, she engages quarterly planning (estimated payments now account for PTET credits and harvesting). Over time, these relatively small annual improvements compound to a significant enhancement of her net worth. Carol can possibly even retire a couple years sooner than planned, or work less, because her money is working more efficiently for her.

These case studies underscore that whether one is near retirement (like Dr. Brian), at peak career (Dr. Alice), or even mid-career (Dr. Carol), **proactive tax and wealth strategies** can dramatically improve outcomes. The specifics vary, but the consistent theme is **planning ahead and leveraging the tools available** – from entities and retirement plans to smart sale structuring and estate planning – to keep more of what is earned and achieve personal goals.

(Visual suggestion: each case could be accompanied by a simple before-and-after table or chart: e.g., “Tax bill before vs after” or “Projected assets at 65 before vs after strategies” to highlight the improvement. Possibly a timeline graphic for each showing major steps taken. Since case studies are narrative, visuals could be minimal, maybe an icon per case (e.g., case 1 with a piggy bank icon for retirement boost, case 2 with a handshake/building icon for selling practice, case 3 with a chart icon for mid-career growth).)

Visual Suggestions for the Lead Magnet

Effective visuals will make these strategies and comparisons clearer. Here are recommended visuals and where to use them:

- **Table of Contents Page:** A clean layout with maybe small icons for each section (e.g., a shield for asset protection/estate, a graph for investments, etc.) to make the PDF inviting.
- **Executive Summary:** Perhaps an infographic or **pie chart** demonstrating the high tax percentage typical surgeons pay (e.g., 40%+ of income to taxes ¹ ²) versus what could be saved. This can hook the reader by visualizing “this is your income pie, and this big slice is taxes – let’s shrink it.”
- **Section 3 (The Problem: Overpaying Taxes):** A **bar chart or pictogram** comparing a surgeon’s effective tax rate vs. average Americans’. E.g., a bar for “Typical Orthopedic Surgeon ~40% tax” vs “Average US Household ~14% tax” (just illustrative) to show they are in a heavily taxed group ⁸⁸. Also perhaps a graphic of 100 doctors and highlighting “75% feel overtaxed” ⁴ – a simple icon array where 75 out of 100 doctor icons are colored.
- **Section 4 (Cost of Inaction):** A **before-and-after wealth projection chart**. For example, a line or bar chart showing “Wealth at Retirement” for Surgeon A (no strategies) vs. Surgeon B (with strategies) – where Surgeon B’s bar is much higher because they saved X in taxes each year and invested it. Or a **lifestyle impact illustration**: two side-by-side cartoon scenes, one of a retired surgeon traveling in comfort (with extra \$40K/year because they planned) vs. one counting pennies (because they lost that to taxes). Perhaps a **stacked bar** showing “Total earnings over career” and how much went to taxes vs. kept – and how planning tilts that.
- **Section 5 (Strategy Framework):** A **timeline graphic** showing the shift in strategy focus from early career to near retirement. For example: Years 1-30 (focus on earning & basic saving) -> Last 10 years (focus on tax optimization, exit, preservation). Could highlight key moves to make in that final decade (maximize contributions, plan sale, do Roth conversions, update estate plans). This gives an overview of phases.
- **Strategy 1 (Entity & Income Structuring):** A **diagram of income flows** comparing a sole proprietor vs. S-Corp. For sole prop: one bucket of \$100K, all hit with 15.3% payroll tax. For S-Corp: split into salary bucket (taxed payroll) and distribution bucket (no payroll tax) ¹⁸, showing how the distribution portion escapes that tax. Also an **organizational chart** showing a practice entity and a separate LLC for real estate, arrow showing rent paid (deductible) to realty LLC, which flows to owner maybe at lower tax (illustrating splitting assets). Could also use a **before vs after table** listing: “Before – taxed on 100% of profit as ordinary; After – part of profit now dividend (saving Medicare tax), SALT workaround applied, etc.” – summing up savings.
- **Strategy 2 (Retirement & Tax-Deferred):** A **comparison chart of contribution limits**: maybe a table with columns for “Regular 401(k)” vs “401k + Cash Balance” for a 60-year-old, showing how one can put ~\$67K vs. ~\$300K ⁸⁹. Also perhaps a small **line chart** showing projected account balance if contributing \$300K/year vs \$60K/year for 5 years – demonstrating the jump. A **simple flowchart** could illustrate a Backdoor Roth process: personal contribution -> Traditional IRA -> immediate

conversion -> Roth IRA (depicting it avoids the normal Roth income limits). Maybe a **timeline** with "Now: high tax, defer max; Retirement: lower tax, consider Roth conversions, distributions".

- **Strategy 3 (Investment Tax Optimization):** Could include a **three-bucket image** representing Asset Location: one bucket labeled "Taxable" (with things like stock index funds, muni bonds), one "Tax-Deferred IRA" (with bonds, REITs), one "Roth" (with high growth stocks) – illustrating what goes where ³⁸ ³⁷. Also a **line graph** showing after-tax portfolio growth with and without asset location – the one with optimization slightly higher over time (this might be subtle but makes the point of compounding benefit ³⁵). For tax-loss harvesting, maybe a **diagram**: stock purchased at \$100 -> drops to \$70 -> sell (harvest \$30 loss) -> buy similar stock -> later stock goes up -> offset future gain. This can be a step-by-step arrows graphic. A **U.S. map** highlighting Opportunity Zones might be too detailed; instead, maybe an icon of a city skyline with a sign "Opportunity Zone – invest gain here, pay tax later" as a call-out. A small chart showing capital gains rates vs. ordinary rates could also reinforce why capital gain = good (20%) vs ordinary 37%.
- **Strategy 4 (Practice Exit & Pre-Liquidity):** A **flowchart** of practice sale deal: Buyer -> pays \$X for tangible assets (tax ordinary) + \$Y for goodwill (tax capital gain) ⁹. If personal goodwill: Buyer -> pays Dr. for personal goodwill (cap gain) ¹³; separate arrow -> pays Corp for other assets (double tax). Illustrate how splitting goodwill saves tax. Also a **bar chart** comparing "Tax if sale structured poorly" vs "Tax if structured optimally" for a hypothetical sale price (like Dr. Brian's \$2M: \$736K vs \$568K tax in case study, visually showing \$168K saved). For installment sale, perhaps an image of a cash flow timeline, highlighting "spread income -> lower brackets". For CRT, a simple diagram: Asset -> CRT (no immediate tax on sale) -> income stream to Dr. -> remainder to charity. For 1031 exchange: a picture of one building arrow exchanging into another building with "no tax" stamp on arrow ⁴⁴. Possibly an **estate planning timeline** if relevant (like plan 5+ years before sale for entity changes).
- **Strategy 5 (Legacy & Estate):** Maybe a **U.S. map** marking federal exemption \$13.99M ⁶³ and listing states with their own estate taxes (NY, MA, IL, etc) and exemptions ⁶⁸ ⁷⁰ – to raise awareness. A **stacked bar** to show estate breakdown: e.g., estate \$20M -> pie slices: 40% tax, 60% to heirs if no plan, vs. 0% tax, 100% heirs if planned under exemption. Perhaps a graphic of an **hourglass or cliff** illustrating NY's cliff: an estate just over exemption tumbling off a cliff (denoting loss of entire exemption) ⁶⁹. A **family tree graphic** to show generation-skipping: Dr. leaves assets to a trust for children & grandchildren – arrow skipping a generation of tax. Or a **shield icon** representing trusts protecting assets. An **ILIT visual**: life insurance policy icon outside a box labeled "taxable estate" meaning it's out. Possibly a timeline if explaining portability or exemption changes (like note "2026 exemption was to drop to ~\$6M ⁹⁰, now \$14M stays – but always subject to politics").
- **Case Studies:** For each case, a **summary graphic** could be beneficial. For Dr. Alice: a bar chart showing "Taxable Income Before (\$1.1M) vs After (\$750K)" and perhaps "Retirement balance by 65 Before vs After" as taller bar. For Dr. Brian: maybe a pie or bar showing how the \$3M of combined sales is allocated (with vs without planning, indicating tax saved and charity portion). Or a flow of money: \$3M sale -> before: \$X to IRS, \$Y to family; after: \$X' to family, \$ to CRT/charity, \$ much smaller to IRS. For Dr. Carol: maybe a bullet list next to a piggy bank graphic enumerating each savings annually (S-corp \$2K, SALT \$22K, etc) summing to "Extra \$50K/year saved." Could use a small chart for one element like SALT: bar showing itemized deduction before (\$10K cap) vs after (\$60K via PTET) to visualize regained deduction.

- **Summary Checklist:** Possibly use icon bullets (e.g., a question mark icon next to each question). Alternatively, a **checklist graphic** with boxes ticked, or an image of a doctor pondering with question bubbles around. But since it's text, maybe just nicely formatting the questions with bold and maybe an icon of a checklist at the side.

Throughout, keep visuals consistent in style (colors, iconography – perhaps using a theme color like a calming blue/green common in medical finance materials). Ensure any graphs are labeled clearly and source any numbers if needed (though likely generalized or from the text).

Since the user mentioned styled PDF design, visuals should be high-level and illustrative rather than extremely detailed technical charts, to maintain readability and professional look.

In summary, a combination of **infographics, charts, and diagrams** will cater to different learning styles – simplifying complex concepts like tax strategy into digestible visuals that complement the written content. Each strategy and case can thus be reinforced with a picture worth a thousand words.

(Reminder: actual images or embed code are not inserted here as this is a planning of suggestions for a designer.)

Summary Checklist: Questions to Ask Your Advisor If You're Within 10 Years of Retirement

As you approach retirement (roughly 5–10 years out), use this checklist to spark critical conversations with your financial and tax advisors. These questions ensure you're covering the key strategies outlined in this guide:

- **Entity Structure & Income:** "Is my current practice structure the most tax-efficient? Should I consider an S-Corp election or other entity changes to reduce payroll taxes ¹⁸ ? Am I paying myself a reasonable salary vs. distribution from the practice ¹⁸ ? Are we taking advantage of any pass-through entity tax (SALT cap workaround) in our state ²⁴ ?"
- **Maximizing Tax-Deferred Savings:** "Can I save more for retirement on a pre-tax or Roth basis given my high income? For instance, would adding a Cash Balance Pension Plan or defined benefit plan make sense ²⁷ ? Am I fully utilizing catch-up contributions and backdoor Roth strategies each year?"
- **Advanced Plan Features:** "Does our 401(k) allow after-tax contributions and in-service Roth conversions (mega backdoor Roth) ³³ ? If not, is it worth amending the plan? Are there 457(b) or deferred comp plans I can use through any hospital affiliation ¹⁴ ?"
- **Investment Tax Efficiency:** "Is my investment portfolio structured for tax efficiency ³⁵ ? Can we review asset location – which assets are in my taxable vs. tax-sheltered accounts ³⁷ ? Are we doing tax-loss harvesting annually ⁴² ? How can we minimize the tax on any large capital gains I anticipate (from rebalancing, stock options, sale of a property, etc.)?"
- **Practice Exit Planning:** "If I plan to sell my practice or shares in X years, what steps should I take now? How can we **structure the sale** to maximize capital gains treatment ⁹ (e.g., allocating to goodwill or personal goodwill ¹³) and minimize ordinary income? Should I get a valuation and start

pre-negotiating terms that are tax-favorable? Would an installment sale or earn-out help spread the tax hit?"

- **Real Estate & Big Assets:** "Do I own my office or other appreciated assets that I might sell around retirement? If so, should we consider a 1031 exchange ⁴⁴ to defer taxes or a Charitable Remainder Trust ⁵⁰ to mitigate the tax and generate income? What's the plan to handle my accounts receivable or any deferred income when I retire – can we time that to minimize taxes?"
- **State Relocation and Taxes:** "Do I plan to retire in a different state? If yes, how can we coordinate that move to reduce state tax on retirement distributions or the sale of my practice (for example, establishing residency in a no-tax state at the right time)? ⁷² "
- **Roth Conversion Gameplan:** "Once I retire (or in a low-income gap year), should we execute Roth conversions ¹⁵ ? What is our plan to manage RMDs and possibly convert IRA funds to Roth in the early retirement years when my tax bracket might be lower? ¹⁴ "
- **Insurance and Asset Protection:** "Are my insurance policies (professional liability, umbrella, etc.) and asset titling optimized to protect my wealth as I transition to retirement? (Not directly tax, but critical for preserving wealth – e.g., homestead, trusts, etc.)"
- **Estate Plan & Gifting:** "Do I need to update my will and estate plan given current \$13.99M exemption ⁶³ ? Should we start **gifting assets** now to reduce future estate tax (such as using the \$19K annual exclusion or larger lifetime gifts) ⁷⁴ ? Would a Spousal Lifetime Access Trust or irrevocable life insurance trust benefit my situation to use my exemption and protect assets for heirs? ¹³ And are all my beneficiary designations (IRA, 401k, life insurance) up-to-date and aligned with the estate plan?"
- **Charitable Goals:** "Do I have charitable or legacy goals we should integrate? For instance, setting up a donor-advised fund or charitable trust to get deductions now and donate over time ⁸⁰ . How can I donate appreciated assets instead of cash to be more tax-efficient ⁴⁹ ? And later, should I plan for Qualified Charitable Distributions from my IRA at 70½ to satisfy RMDs tax-free?"
- **Cash Flow in Retirement:** "Can we project my retirement income needs and sources and see how to withdraw in a tax-savvy way? Which accounts (taxable vs. IRA vs. Roth) should we tap first to minimize taxes over the long run? ³⁵ This includes considering the taxability of Social Security and Medicare IRMAA brackets – how do we keep my taxable income in check to avoid high Medicare premiums, if applicable?"
- **Contingency Planning:** "If tax laws change (like estate exemption dropping or new surcharges), do we have a plan B? For example, if the estate exemption were cut in half, what would we do – additional gifting, SLATs, etc.? If income tax rates rise, should we accelerate certain income or Roth conversions now?"

Each of these questions opens dialogue on critical aspects of your financial plan. A great advisor should address them transparently, tailoring answers to your situation. If you get unclear or unfavorable responses (or notice opportunities being overlooked), it may be a signal to seek a second opinion. By asking these questions, you're effectively conducting a thorough check-up on your financial health – much like you would

for a patient before a major procedure. The result should be a coordinated strategy for your final working years and transition into a financially secure, tax-efficient retirement.

Disclaimer: *The strategies and examples in this guide are for educational purposes and general guidance. Every surgeon's financial situation is unique. Before implementing any tax or investment strategy, consult with qualified professionals (CPA, CFP, attorney) who can tailor recommendations to your personal circumstances and the most current laws. Tax laws (federal and state) are subject to change, and strategies must comply with all IRS rules to be effective. Nothing in this guide constitutes specific tax, legal, or investment advice. Implementing advanced strategies may require professional oversight to ensure proper execution and to understand potential risks (such as market risk, legislative changes, or audit considerations). Ultimately, informed planning can yield significant benefits, but individual due diligence and professional advice are indispensable.*

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